Stillwater content review packet

For a licensed therapist reviewing a draft suicide prevention website for adults and young people in the United States.

> **Draft. Not reviewed.** An AI assistant drafted the website text, and one founder supplied the research brief. No licensed clinician has reviewed any of it, and none of it should be relied on until a review is recorded. Reviewer: not yet recorded. Review date: not yet recorded.

Built November 6, 2026 from the website files and project notes. Fingerprint cb99fa0a4daa. If the pages change, this packet must be rebuilt with `python3 tools/make_review_packet.py`, and numbers may change.

How to use this packet

What's inside:

- Part 1: Text that appears on every page
- Part 2: Each English page, sentence by sentence, with its source links
- Part 3: The founder-supplied research brief
- Part 4: Crisis resources, and what each button does
- Part 5: Questions written for you
- Part 6: How the research was read, and how the pages compare with the brief
- Part 7: Proposed shorter sentences, optional
- Part 8: Decisions and sign-off

The Spanish pages are in a separate packet for a fluent reader.

How to record feedback:

- **Mark each section.** Each section ends with a box: OK as written, OK with changes, or Not for the website.
- **Refer to sentences by number.** Every sentence has a number, such as HELP-12. In the Notes line, write something like "HELP-12: change to ...". Writing on the page is fine.
- **Repeated sentences.** A gray sentence marked "same as" repeats an earlier one. A decision on the first applies to all of them.
- **Answer the questions in Part 5** if you can. Skip any that aren't yours to answer.
- **Finish with Part 8.** It records your decisions, your role, and the date. Please don't write personal contact details in this packet. It may be stored in a public project.

You can use a pen on paper, or add notes to the PDF in a PDF app. Return it to the founder who gave it to you.

If anything here could cause harm, please say so first. Use this box, or tell the founders right away, for anything that should come down or change before anything else happens.

You can see the draft website as it looks on a screen if a founder shares a preview link. The draft pages are not on the public website yet. Parts of an older version are public at https://stillwatersps.github.io/suicideprevention/.

Part 1: Text that appears on every page

The crisis strip, header, crisis buttons, and footer. Each page has one version of these. Different wording on different pages is listed here once.

SHARED–1 Need help now?

SHARED–2 Call 988 or text 988 any time.

SHARED–3 In immediate danger, call 911.

Call 988: `tel:988`
text 988: `sms:988`
call 911: `tel:911`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SHARED–4 **About Stillwater**

SHARED–5 Stillwater is an early project to help people in the United States find suicide prevention support.

SHARED–6 Stillwater is not a crisis service.

SHARED–7 The call, text, and chat links on this site connect you to outside services, such as the 988 Suicide & Crisis Lifeline.

SHARED–8 If you or someone else is in immediate danger, call 911.

SHARED–9 Privacy: this site has no sign-ups, forms, analytics, or cookies.

SHARED–10 It is hosted on GitHub Pages, and GitHub says it logs and stores visitors' IP addresses for security.

SHARED–11 Quick exit and saved offline copies have limits.

SHARED–12 Read about privacy and what this site can't hide.

SHARED–13 Early preview.

SHARED–14 A suicide prevention professional has not reviewed this content yet.

SHARED–15 An AI assistant compared crisis numbers and links with each provider's website on November 6, 2026.

SHARED–16 No person has confirmed them or tried them on a phone.

SHARED–17 See the review status of each page.

SHARED–18 Photo: Joshua tree at sunset in Queen Valley, by Emily Hassell, National Park Service (public domain), via Wikimedia Commons.

GitHub says it logs and stores visitors' IP addresses: `https://docs.github.com/en/pages/getting-started-with-github-pages/what-is-github-pages#data-collection`
Wikimedia Commons: `https://commons.wikimedia.org/wiki/File:Joshua_tree_at_sunset_in_Queen_Valley_(50601665443).jpg`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SHARED–19 **Need to talk to someone now?**

SHARED–20 • Call 988Talk to a counselor

SHARED–21 • Text 988Text a counselor

SHARED–22 • Chat OnlineWeb chat with 988

SHARED–23 The 988 Suicide & Crisis Lifeline is free, confidential, and available any time.

SHARED–24 It is a separate service from Stillwater.

SHARED–25 On a device that can't call or text?

SHARED–26 Call or text 988 from a phone, or go to 988lifeline.org.

SHARED–27 *same as SHARED-3:* In immediate danger, call 911.

SHARED–28 Ayuda en español

SHARED–29 Save 988 to your contacts

SHARED–30 This opens a small contact card for the 988 Lifeline.

SHARED–31 You choose whether to save it, and nothing is sent to Stillwater.

SHARED–32 On a shared phone, other people may see a saved contact.

SHARED–33 *same as SHARED-23:* The 988 Suicide & Crisis Lifeline is free, confidential, and available any time.

SHARED–34 You can contact it about someone else, too.

SHARED–35 *same as SHARED-24:* It is a separate service from Stillwater.

SHARED–36 *same as SHARED-23:* The 988 Suicide & Crisis Lifeline is free, confidential, and available any time.

SHARED–37 *same as SHARED-24:* It is a separate service from Stillwater.

SHARED–38 You don't need research to reach out.

SHARED–39 Trained counselors at the 988 Suicide & Crisis Lifeline answer calls, texts, and chats.

SHARED–40 *same as SHARED-24:* It is a separate service from Stillwater.

Call 988Talk to a counselor: `tel:988`
Text 988Text a counselor: `sms:988`
Chat OnlineWeb chat with 988: `https://chat.988lifeline.org/`
988lifeline.org: `https://988lifeline.org/`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Part 2: English pages

Text from the crisis strip, header, crisis buttons, and footer is in Part 1. Folded sections are shown open. Links show their full address, so they can be checked from paper.

Page: Home

Address once published: https://stillwatersps.github.io/suicideprevention/. Numbers start with HOME.

HOME-1 Find suicide prevention support

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

HOME-2 **You are not alone.**

HOME-3 If you're feeling overwhelmed, in crisis, or having thoughts of suicide, the 988 Suicide & Crisis Lifeline is there for you — anytime, day or night.

- HOME-4 • *same as SHARED-20:* Call 988Talk to a counselor
- HOME-5 • *same as SHARED-21:* Text 988Text a counselor
- HOME-6 • *same as SHARED-22:* Chat OnlineWeb chat with 988

HOME-7 *same as SHARED-25:* On a device that can't call or text?

HOME-8 *same as SHARED-26:* Call or text 988 from a phone, or go to 988lifeline.org.

HOME-9 These connect you to the 988 Suicide & Crisis Lifeline, not to Stillwater.

HOME-10 *same as SHARED-28:* Ayuda en español

HOME-11 Image description: A Joshua tree stands in front of a pink and blue sunset sky in the desert.

- HOME-12 • Your privacyStillwater has no sign-ups, forms, or tracking.
- HOME-13 • Our web host logs visitors' IP addresses.
- HOME-14 • 988 is available 24/7Trained crisis counselors answer calls, texts, and chats.
- HOME-15 • For you and your loved onesReach out for yourself or for someone you're worried about.

HOME-16 Find the support you need

Call 988Talk to a counselor: `tel:988`
Text 988Text a counselor: `sms:988`
Chat OnlineWeb chat with 988: `https://chat.988lifeline.org/`
988lifeline.org: `https://988lifeline.org/`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

HOME-17 **Where would you like to start?**

HOME-18 Whether you're in crisis, looking for ways to cope, or supporting someone you care about, these pages can point you to support.

HOME-19 **I'm having a hard time**

HOME-20 Find support for difficult moments, and learn what getting help looks like.

HOME-21 **I'm worried about someone**

HOME-22 Learn how to support a friend, family member, or loved one through it.

HOME-23 **I want to learn more**

HOME-24 See what research shows about suicide prevention, and where the evidence is still thin.

HOME-25 **Not sure what you're feeling?**

HOME-26 Start here, at your own pace.

HOME-27 What you open isn't saved, and there's no score.

HOME-28 **I've lost someone to suicide**

HOME-29 What you may feel, where to find support, and how to talk with children.

HOME-30 You don't have to face this alone.

HOME-31 Support is available, whenever you're ready.

HOME-32 Call or text 988, or chat at chat.988lifeline.org.

Call: `tel:988`
text 988: `sms:988`
chat.988lifeline.org: `https://chat.988lifeline.org/`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Page: Get Help

Address once published: https://stillwatersps.github.io/suicideprevention/get-help/. Numbers start with HELP.

HELP-1 For you

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

HELP-2 ## Getting help for yourself

HELP-3 If you're struggling or having thoughts of suicide, you don't have to handle it alone.

HELP-4 Here are ways to reach someone and what to expect.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

HELP-5 **Right now**

- HELP-6 • Reach out to 988 with the call, text, or chat buttons above.
- HELP-7 • You don't need to be sure it's an emergency.

HELP–8 • It's free and confidential.

HELP–9 • In immediate danger?

HELP–10 • Call 911.

HELP–11 • If one way doesn't connect, try another: call, text, or chat.

HELP–12 • The rest of this page can wait.

HELP–13 • Open a section below when you're ready.

HELP–14 Draft for review.

HELP–15 A suicide prevention professional has not reviewed this page yet.

HELP–16 An AI assistant drafted it and compared the phone numbers and links with each provider's website on November 6, 2026.

HELP–17 *same as SHARED-16:* No person has confirmed them or tried them on a phone.

Call 911: `tel:911`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

HELP–18 **What happens when you reach out to 988**

HELP–19 *same as HELP-7:* You don't need to be sure it's an emergency.

HELP–20 The 988 Lifeline says anyone who is struggling can reach out, whether you are depressed, need to talk, or are thinking about suicide.

HELP–21 It also takes concerns about alcohol or drug use.

HELP–22 **Show moreShow less**

HELP–23 **If you call**

HELP–24 You'll hear a short recorded message with options, such as pressing 1 for the Veterans Crisis Line, 2 for Spanish, or 3 for LGBTQ+ youth.

HELP–25 You don't have to press anything.

HELP–26 If you stay on the line, you'll be connected to a local 988 center.

HELP–27 Hold music plays while you are connected.

HELP–28 A counselor then says hello, asks if you are safe, listens to what's going on, and shares resources that may help.

HELP–29 **If you text**

HELP–30 Send any message to 988.

HELP–31 Automated messages come first.

HELP–32 Reply NEXT to accept the terms of service, then answer a few questions that help the counselor know how to support you.

HELP–33 You may then get a message asking you to wait.

HELP–34 A counselor then says hello, asks if you are safe, and listens.

HELP–35 **If you chat online**

HELP–36 The chat opens with a short form that helps the counselor know how to support you.

HELP-37 It asks for your zip code, whether you are having thoughts of suicide, and how upset you feel.

HELP-38 A phone number is optional.

HELP-39 Starting the chat accepts the terms of service.

HELP-40 You then wait to be connected to a counselor.

HELP-41 **What it costs**

HELP-42 Contacting 988 is free, and you don't need insurance.

HELP-43 Your phone carrier may still charge for calls or texts if your plan doesn't include unlimited use.

HELP-44 If a counselor sends emergency services, you may get a bill, for example for an ambulance or hospital care.

HELP-45 Services they refer you to may charge too, so ask about fees first.

HELP-46 **If you can't get through**

HELP-47 Try another way.

HELP-48 If a call doesn't connect, try text or chat.

HELP-49 If texting is busy, try a call or chat.

HELP-50 In an emergency, call 911.

HELP-51 **What stays private**

HELP-52 The 988 Lifeline says conversations are confidential, but contacting it isn't anonymous.

HELP-53 You don't have to give your name or your exact location to get support.

HELP-54 The counselor has your phone number if you call or text, or your IP address if you chat.

HELP-55 In rare emergencies they must give this to 911, and 911 may ask your phone or internet company for your exact location.

HELP-56 Counselors take notes, and centers keep records for at least three years.

HELP-57 Call recordings or chat and text transcripts may be used to improve the service.

HELP-58 You can ask a counselor not to share your personal information.

HELP-59 The Lifeline's confidentiality statement lists when information can be shared, including when someone is at imminent risk of harm, when the law requires it, or when you agree.

HELP-60 On a shared phone or computer, visits to this site can show up in browser history, and calls or texts can show up in phone logs.

HELP-61 Whether a call appears on a phone bill depends on the carrier.

HELP-62 *same as SHARED-11:* Quick exit and saved offline copies have limits.

HELP-63 See what they can and can't hide.

HELP-64 **When emergency services get involved**

HELP-65 The 988 Lifeline is separate from 911, and SAMHSA says most crises are managed by counselors working with you, without involving law enforcement.

HELP-66 The Lifeline recommends that counselors contact emergency services, which can mean 911, police, or a sheriff, only if someone is in danger of being hurt right now or very soon and they can't work out a safer plan together.

HELP-67 It reports that fewer than 2 in 100 calls involve emergency services, and that more than half of those happen with the caller's consent.

HELP-68 This is a recommendation, not a guarantee.

HELP-69 The Lifeline's terms say that if a counselor believes there is an immediate threat to someone's physical safety, you may not get a say: they may ask for 911 or a mobile crisis team to come to your location, and can share your information without your permission.

HELP-70 If you need help because of an immediate threat to your safety, the terms ask you to call 911 first.

HELP-71 These details come from the 988 Lifeline's website and SAMHSA, and can change.

HELP-72 Stillwater does not run 988 and cannot see who contacts it.

confidentiality statement: `https://988lifeline.org/confidentiality/`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

HELP-73 **Other ways to reach someone**

HELP-74 These are separate services with their own rules.

HELP-75 Stillwater does not run them.

HELP-76 **En español**

HELP-77 Para hablar con un consejero de la Línea 988 en español, marca 988 y luego presiona 2.

HELP-78 Para enviar un mensaje de texto, envía la palabra AYUDA al 988.

HELP-79 También puedes chatear en español.

HELP-80 Es gratis y confidencial, y está disponible las 24 horas.

HELP-81 La Línea 988 es un servicio aparte de Stillwater.

HELP-82 Si hay peligro inmediato, llama al 911.

HELP-83 Llamar al 988 | Enviar AYUDA al 988 | Chatear en español

HELP-84 **Deaf or hard of hearing**

HELP-85 You can reach 988 counselors in American Sign Language by dialing 988 from a videophone (VP) number.

HELP-86 You can also text or chat.

HELP-87 TTY users can use a preferred relay service or dial 711, then 988.

HELP-88 988 options for Deaf and hard of hearing people

HELP-89 **Other languages**

HELP-90 Call 988 and ask for an interpreter, or say the name of your language.

HELP-91 The 988 Lifeline offers more than 240 languages this way, on voice calls only.

HELP-92 Call 988

HELP-93 **Veterans Crisis Line**

HELP-94 For veterans, service members, National Guard and Reserve members, and the people who support them.

HELP-95 Free, confidential, and available 24/7.

HELP-96 You don't need to be enrolled in VA benefits or health care.

HELP-97 Call 988, then press 1 | Text 838255 | Veterans Crisis Line chat

HELP-98 **LGBTQ+ youth, through 988**

HELP-99 The 988 Lifeline says its specialized service for LGBTQ+ youth has been restored.

HELP-100 It is separate from The Trevor Project.

HELP-101 Call 988, then press 3 | Text PRIDE to 988

HELP-102 **The Trevor Project**

HELP-103 For LGBTQ+ young people.

HELP-104 Its terms describe the phone lifeline as for people under age 25.

HELP-105 Trained counselors by phone, text, or chat, free and confidential, 24/7.

HELP-106 Message and data rates may apply to texts.

HELP-107 On a phone, The Trevor Project suggests calling or texting, and using chat from a computer.

HELP-108 Call 1-866-488-7386 | Text START to 678-678 | Trevor Project chat and TTY

HELP-109 **Crisis Text Line**

HELP-110 Trained volunteer crisis counselors by text, free and confidential, 24/7, anywhere in the United States.

HELP-111 Messages are free on AT&T, T-Mobile, Sprint, and Verizon.

HELP-112 On other carriers, standard rates apply and 741741 can appear on the bill.

HELP-113 It says many T-Mobile prepaid plans don't allow numbers like 741741, so if a text doesn't go through, try another way.

HELP-114 Text HOME to 741741 | Crisis Text Line website

HELP-115 **911**

HELP-116 *same as SHARED-8:* If you or someone else is in immediate danger, call 911.

HELP-117 Texting 911 only works in some places, and the FCC advises making a voice call if you can.

HELP-118 If you text 911 where it isn't available, you should get an automatic message telling you to use another way.

HELP-119 If you are Deaf, hard of hearing, or can't speak and texting 911 isn't available, use a TTY or relay service if you can.

HELP-120 *same as HELP-10:* Call 911

Llamar al 988: `tel:988`

Enviar AYUDA al 988: `sms:988`

Chatear en español: `https://chat.988lifeline.org/?lang=es`

988 options for Deaf and hard of hearing people: `https://988lifeline.org/deaf-hard-of-hearing-hearing-loss/`

Text 838255: `sms:838255`

Veterans Crisis Line chat: `https://www.veteranscrisisline.net/get-help-now/chat/`

Call 1-866-488-7386: `tel:+18664887386`

Text START to 678-678: `sms:678678`

Trevor Project chat and TTY: `https://www.thetrevorproject.org/get-help/`

Text HOME to 741741: `sms:741741`

Crisis Text Line website: `https://www.crisistextline.org/`

Call 911: `tel:911`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

HELP–121 **Support over time**

HELP–122 A crisis line can help with a hard moment.

HELP–123 Longer-term care can help with what is behind it.

HELP–124 The National Institute of Mental Health (NIMH) says effective treatments are available for people at risk of suicide.

HELP–125 These include kinds of talk therapy, such as cognitive behavioral therapy and dialectical behavior therapy.

HELP–126 The American Foundation for Suicide Prevention says recovery is possible with evidence-based treatment for depression and other mental health conditions.

HELP–127 **Show moreShow less** *same as HELP-22*

HELP–128 • Talk to a health care provider.

HELP–129 • If you have a doctor or nurse you see, you can bring up your mental health yourself, and they can refer you to a specialist.

HELP–130 • NIMH has tips for that conversation.

HELP–131 • Search for care near you.

HELP–132 • SAMHSA describes FindTreatment.gov as a confidential and anonymous treatment finder.

HELP–133 • Call for a referral.

HELP–134 • SAMHSA's National Helpline, 1-800-662-4357, offers free treatment referrals and information, 24/7, in English and Spanish.

HELP–135 • It does not provide counseling and is not a crisis line.

HELP–136 • For Spanish, choose the option to speak with a national representative.

HELP–137 • TTY: 1-800-487-4889.

tips for that conversation: `https://www.nimh.nih.gov/health/publications/tips-for-talking-with-your-health-care-provider`

FindTreatment.gov: `https://findtreatment.gov/`

1-800-662-4357: `tel:+18006624357`

1-800-487-4889: `tel:+18004874889`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

HELP–138 **Planning for a hard moment**

HELP–139 A safety plan is a short written list of what to do when things get hard, made ahead of time so you don't have to work it out during a crisis.

HELP–140 A therapist or other mental health professional can help you make one.

HELP–141 A plan does not replace professional care.

HELP-142 **Show moreShow less** *same as HELP-22*

HELP-143 NIMH says research has shown that safety plans can help lower a person's risk of acting on suicidal thoughts.

HELP-144 Some of this research paired the plan with clinician contact and follow-up, so it doesn't show what a plan made alone would do.

HELP-145 For children and teens, a review of brief programs built around a safety plan found no clear benefit.

HELP-146 See what the research shows about safety plans.

HELP-147 A safety plan usually includes:

HELP-148 • Your own warning signs that things are getting harder

HELP-149 • Things you can do on your own to get through it

HELP-150 • People and places that take your mind off it

HELP-151 • Family or friends you can reach out to

HELP-152 • Professionals and crisis lines you can contact

HELP-153 • Ways to make your surroundings safer

HELP-154 You don't have to work through a list before asking for help.

HELP-155 You can call or text 988, or call 911 in an emergency, at any point.

HELP-156 The developers of the Stanley-Brown Safety Plan offer a form (PDF) for personal use.

HELP-157 Stillwater does not see or save what you write on it.

HELP-158 A copy on a shared device, or on paper, could be seen by others, so keep it somewhere you feel safe.

a form (PDF): `https://suicidesafetyplan.com/wp-content/uploads/2024/12/Stanley-Brown-Safety-Plan-05-02-2024.pdf`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

HELP-159 ## If you're under 18

HELP-160 You can call or text 988 yourself, even if there is no adult you can tell.

HELP-161 Its rules don't set a minimum age, and you don't have to give your name or age to get support.

HELP-162 It can help to save 988 in your phone now, in case you need it later.

HELP-163 More for teens: reaching out, trusted adults, and privacy

HELP-164 **Show moreShow less** *same as HELP-22*

HELP-165 It can also help to tell an adult you trust, such as a parent or guardian, a relative, a teacher, a coach, a spiritual leader, or a doctor.

HELP-166 It's okay to ask what they can keep private first.

HELP-167 If you tell a school counselor you are thinking about suicide, expect them to tell a parent or guardian.

HELP-168 Their national association treats this as required, even for students who are 18.

HELP-169 If a parent's abuse or neglect is the reason, the counselor contacts child protective services instead.

HELP-170 Crisis lines keep conversations confidential, with limits.

HELP-171 The 988 Lifeline says it may share information with 911, police, or a mobile crisis team, without your permission, in some situations.

HELP–172 For example, if someone is in danger of being hurt very soon, or if a young person is being abused or neglected under state law.

HELP–173 The Trevor Project and Crisis Text Line have similar limits.

HELP–174 The Trevor Project says it won't tell parents or guardians unless the law or a subpoena requires it.

HELP–175 A small number of crisis centers bill insurance and may ask for your details.

HELP–176 You can say no and still get support.

HELP–177 If you give them and a claim is filed, a parent or guardian could get an insurance statement.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

HELP–178 ## More on this site

HELP–179 Not sure what you're feeling? | After a suicide loss | For teens | About and privacy

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

HELP–180 ## Sources

HELP–181 Checked November 6, 2026.

HELP–182 An AI assistant drafted these summaries for Stillwater, and no qualified person has reviewed them yet.

HELP–183 The organizations listed here have not reviewed or endorsed them.

HELP–184 **Show sources (28)**

HELP–185 • What to Expect, 988 Suicide & Crisis Lifeline

HELP–186 • Get Help, 988 Suicide & Crisis Lifeline

HELP–187 • Texting the 988 Lifeline (FAQ), 988 Suicide & Crisis Lifeline

HELP–188 • Calling the 988 Lifeline (FAQ), 988 Suicide & Crisis Lifeline

HELP–189 • Other languages (FAQ), 988 Suicide & Crisis Lifeline

HELP–190 • Deaf, Hard of Hearing, Hearing Loss, 988 Suicide & Crisis Lifeline

HELP–191 • Confidentiality, 988 Suicide & Crisis Lifeline

HELP–192 • Terms of Service, 988 Suicide & Crisis Lifeline

HELP–193 • Police intervention (FAQ), 988 Suicide & Crisis Lifeline

HELP–194 • Tracing callers, texters, and chatters (FAQ), 988 Suicide & Crisis Lifeline

HELP–195 • 988 Lifeline chat.

HELP–196 • The form was read but not submitted.

HELP–197 • 988 Frequently Asked Questions, Substance Abuse and Mental Health Services Administration (SAMHSA)

HELP–198 • National Helpline, SAMHSA

HELP–199 • FindTreatment.gov, SAMHSA

HELP–200 • For Young People Looking for Help, SAMHSA

HELP-201 • Veterans Crisis Line, U.S. Department of Veterans Affairs

HELP-202 • Get Help, The Trevor Project

HELP-203 • Terms of Service, The Trevor Project

HELP-204 • FAQ, Crisis Text Line

HELP-205 • Privacy Policy, Crisis Text Line

HELP-206 • Text to 911: What You Need to Know, Federal Communications Commission

HELP-207 • Frequently Asked Questions About Suicide, National Institute of Mental Health

HELP-208 • Tips for Talking With a Health Care Provider, National Institute of Mental Health

HELP-209 • Child and Adolescent Mental Health, National Institute of Mental Health

HELP-210 • After an attempt, American Foundation for Suicide Prevention

HELP-211 • Statement on the Importance of Antidepressants and Evidence-Based Treatment, American Foundation for Suicide Prevention

HELP-212 • Stanley-Brown Safety Planning Intervention

HELP-213 • The School Counselor and Suicide Risk Assessment, American School Counselor Association

What to Expect: https://988lifeline.org/get-help/what-to-expect/

Get Help: https://988lifeline.org/get-help/

Texting the 988 Lifeline (FAQ): https://988lifeline.org/faq/texting-the-988-lifeline/

Calling the 988 Lifeline (FAQ): https://988lifeline.org/faq/calling-the-988-lifeline/

Other languages (FAQ): https://988lifeline.org/faq/calling-the-988-lifeline/faq-is-the-988-lifeline-available-in-other-languages-for-non-english-speakers/

Deaf, Hard of Hearing, Hearing Loss: https://988lifeline.org/deaf-hard-of-hearing-hearing-loss/

Confidentiality: https://988lifeline.org/confidentiality/

Terms of Service: https://988lifeline.org/terms-of-service/

Police intervention (FAQ): https://988lifeline.org/faq/about-us/faq-does-vibrant-use-police-intervention-for-callers-texters-and-chatters-to-the-988-lifeline/

Tracing callers, texters, and chatters (FAQ): https://988lifeline.org/faq/about-us/faq-does-the-988-lifeline-trace-callers-texters-or-chatters/

988 Lifeline chat: https://chat.988lifeline.org/

988 Frequently Asked Questions: https://www.samhsa.gov/mental-health/988/faqs

National Helpline: https://www.samhsa.gov/find-help/helplines/national-helpline

FindTreatment.gov: https://findtreatment.gov/

For Young People Looking for Help: https://www.samhsa.gov/mental-health/what-is-mental-health/how-to-talk/young-people

Veterans Crisis Line: https://www.veteranscrisisline.net/

Get Help: https://www.thetrevorproject.org/get-help/

Terms of Service: https://www.thetrevorproject.org/terms-of-service/

FAQ: https://www.crisistextline.org/about-us/faq/

Privacy Policy: https://www.crisistextline.org/privacy-policy/

Text to 911: What You Need to Know: https://www.fcc.gov/consumers/guides/what-you-need-know-about-text-911

Frequently Asked Questions About Suicide: https://www.nimh.nih.gov/health/publications/suicide-faq

Tips for Talking With a Health Care Provider: https://www.nimh.nih.gov/health/publications/tips-for-talking-with-your-health-care-provider

Child and Adolescent Mental Health: https://www.nimh.nih.gov/health/topics/child-and-adolescent-mental-health

After an attempt: https://afsp.org/after-an-attempt/

Statement on the Importance of Antidepressants and Evidence-Based Treatment: https://afsp.org/story/afsp-statement-on-the-importance-of-antidepressants-and-evidence-based-treatment

Stanley-Brown Safety Planning Intervention: https://suicidesafetyplan.com/

The School Counselor and Suicide Risk Assessment: https://www.schoolcounselor.org/Standards-Positions/Position-Statements/ASCA-Position-Statements/The-School-Counselor-and-Suicide-Risk-Assessment

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Page: Help Someone

Address once published: https://stillwatersps.github.io/suicideprevention/help-someone/. Numbers start with SOME.

SOME-1 For friends and family

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SOME-2 ### Helping someone you're worried about

SOME-3 If you're worried about a friend, family member, or someone else, you can help.

SOME-4 You don't need to be an expert.

SOME-5 If you think they are in immediate danger, call 911.

SOME-6 If it is safe, stay with them, or make sure they are in a private, secure place with another caring person, until help comes.

SOME-7 If you're worried they may be in danger of suicide, call or text 988.

SOME-8 *same as HELP-14:* Draft for review.

SOME-9 *same as HELP-15:* A suicide prevention professional has not reviewed this page yet.

SOME-10 *same as HELP-16:* An AI assistant drafted it and compared the phone numbers and links with each provider's website on November 6, 2026.

SOME-11 *same as SHARED-16:* No person has confirmed them or tried them on a phone.

call 911: tel:911

call: tel:988

text 988: sms:988

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SOME-12 ### Signs someone may be struggling

SOME-13 These may be signs that someone is thinking about suicide.

SOME-14 Take any of these seriously.

SOME-15 Be especially concerned if a behavior is new, has increased, or seems related to a painful event, loss, or change.

SOME–16 **What they talk about**

- SOME–17 Wanting to die
- SOME–18 Feeling great guilt or shame
- SOME–19 Being a burden to others

SOME–20 **How they feel**

- SOME–21 Empty, hopeless, trapped, or having no reason to live
- SOME–22 Extremely sad, more anxious, agitated, or full of rage
- SOME–23 In unbearable emotional or physical pain

SOME–24 **What they do**

- SOME–25 Making a plan or researching ways to die
- SOME–26 Withdrawing from friends, saying goodbye, giving away important items, or making a will
- SOME–27 Taking dangerous risks
- SOME–28 Having extreme mood swings
- SOME–29 Eating or sleeping more or less
- SOME–30 Using drugs or alcohol more often

SOME–31 If you notice these signs, get help as soon as possible.

SOME–32 You don't have to be certain.

SOME–33 You can contact 988 at any time if you're worried about someone.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SOME–34 **Five steps you can take**

SOME–35 The National Institute of Mental Health (NIMH) publishes these steps, which are also used by the 988 Lifeline's #BeThe1To campaign.

SOME–36 Some details below come from the other sources listed at the end of this page.

SOME–37 **Ask**

SOME–38 Ask directly: "Are you thinking about suicide?

SOME–39 It isn't an easy question, but it can start the conversation.

SOME–40 NIMH says studies show that asking does not increase suicidal thoughts or behavior.

SOME–41 **Be there**

SOME–42 Listen without judgment to learn what they are thinking and feeling.

SOME–43 NIMH says research suggests that acknowledging and talking about suicide may reduce suicidal thoughts.

SOME–44 **Help keep them safe**

SOME–45 Ask whether they have a plan.

SOME–46 If they do, call or text 988 right away.

SOME–47 If they intend to act on it right away and have the means, call 911.

SOME-48 If you can do it safely, remove or safely store anything they could use to hurt themselves.

SOME-49 The 988 Lifeline can also help you work out what to do next.

SOME-50 **Help them connect**

SOME-51 Help them reach 988 by call or text, and other local resources.

SOME-52 You can also help them reach a family member, friend, spiritual advisor, or mental health professional they trust.

SOME-53 You can contact 988 yourself about them, too.

SOME-54 If they are in physical danger, the counselor will contact 911.

SOME-55 **Follow up**

SOME-56 Stay in touch after a crisis or after they leave care.

SOME-57 NIMH says supportive, ongoing contact can play an important role in preventing suicide.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SOME-58 **What helps in the conversation**

SOME-59 This draws on advice from the 988 Lifeline and NIMH.

SOME-60 **Try to**

- SOME-61 • Be direct, and talk openly and calmly about suicide.
- SOME-62 • Listen, let them finish, and accept what they feel.
- SOME-63 • Say back what you hear, such as "It sounds like you're saying..."
- SOME-64 • Let them know help is out there and that you'll help them reach it.
- SOME-65 • Don't rush to say everything will be fine.
- SOME-66 • Get help from people who specialize in crisis support.

SOME-67 **What to avoid**

- SOME-68 • Don't debate whether suicide is right or wrong, or lecture about the value of life.
- SOME-69 • Don't act shocked, or dare them to do it.
- SOME-70 • Don't dismiss what they say as a bid for attention.
- SOME-71 • All talk of suicide should be taken seriously.
- SOME-72 • Don't promise to keep it a secret.
- SOME-73 • If someone tells you about a plan to hurt themselves, tell someone who can help.
- SOME-74 • Don't leave them alone if they tell you they are going to end their life.
- SOME-75 • If it is safe, stay with them and call 911, or call or text 988.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SOME-76 **If it's a child or teen**

SOME-77 **If you're a parent or caregiver**

SOME-78 Take what they say seriously, even if they say they didn't mean it.

SOME-79 Don't argue with their feelings.

SOME-80 Tell them you care and that you will help them get help.

SOME-81 If they have a suicide plan or can't stop thinking about ending their life, call or text 988 right away, and call 911 if they need immediate help.

SOME-82 Stay close and don't leave them alone while you get help.

SOME-83 Remove or safely store anything they could use to hurt themselves.

SOME-84 Any child with suicidal thoughts or plans should be evaluated right away by a trained mental health professional.

SOME-85 Call their pediatrician or a mental health provider now, even if it doesn't seem like a crisis.

SOME-86 Their pediatrician can also help you make a safety plan.

SOME-87 **At school**

SOME-88 School counselors and school psychologists can help connect a family to support.

SOME-89 School counselors are expected to tell a parent or guardian when a student has thoughts of suicide, even at 18.

SOME-90 If a parent's abuse or neglect is the reason, they contact child protective services instead.

SOME-91 So don't promise a young person that it will stay private.

SOME-92 **If you're a young person worried about a friend**

SOME-93 You don't need to have the answers.

SOME-94 Start with a caring question and listen.

SOME-95 If you're worried about suicide, it's okay to ask directly: "Are you thinking about suicide?

SOME-96 Encourage your friend to talk to a trusted adult, such as a parent, coach, teacher, or school counselor, and talk to one yourself.

SOME-97 If your friend talks about suicide or about hurting themselves, don't keep it a secret, even if they ask you to.

SOME-98 Tell a trusted adult, or call or text 988 to ask what to do.

SOME-99 You can contact 988 at any time, even if it's about a friend.

SOME-100 If they say they are going to end their life, call 911 if it's an emergency.

SOME-101 If you're with them and it is safe, stay with them.

SOME-102 If they told you by text or online, keep talking if you can while you get an adult, or call or text 988.

SOME-103 You don't have to handle this by yourself.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SOME-104 **If you see a worrying post online**

SOME-105 If you think someone may be in danger right now, call 911.

SOME-106 You can also contact the site's safety team, because many social media sites have a way to get help for the person posting.

SOME-107 The 988 Lifeline explains how this works on different platforms.

how this works on different platforms: https://988lifeline.org/help-someone-else/support-on-social-media/

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SOME-108 ## Look after yourself too

SOME-109 Supporting someone through a hard time can bring up difficult emotions of your own, and the responsibility doesn't have to be yours alone.

SOME-110 The 988 Lifeline encourages helpers to reach out for support themselves.

SOME-111 You can call or text 988 for yourself, even if you're not the one in crisis.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SOME-112 ## More on this site *same as HELP-178*

SOME-113 After a suicide loss | Not sure what you're feeling? | What the research shows

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SOME-114 ## Sources *same as HELP-180*

SOME-115 *same as HELP-181:* Checked November 6, 2026.

SOME-116 *same as HELP-182:* An AI assistant drafted these summaries for Stillwater, and no qualified person has reviewed them yet.

SOME-117 *same as HELP-183:* The organizations listed here have not reviewed or endorsed them.

SOME-118 ### Show sources (13)

SOME-119 • Warning Signs of Suicide, National Institute of Mental Health

SOME-120 • 5 Action Steps to Help Someone Having Thoughts of Suicide, National Institute of Mental Health

SOME-121 • *same as HELP-207:* Frequently Asked Questions About Suicide, National Institute of Mental Health

SOME-122 • Suicide Prevention, Substance Abuse and Mental Health Services Administration (SAMHSA)

SOME-123 • 988 Frequently Asked Questions, SAMHSA

SOME-124 • Help Someone Else, 988 Suicide & Crisis Lifeline

SOME-125 • Safety and support on social media, 988 Suicide & Crisis Lifeline

SOME-126 • What should I do if my child or teen is thinking about suicide?, American Academy of Pediatrics

SOME-127 • Suicide in Children and Teens, American Academy of Child & Adolescent Psychiatry

SOME-128 • Preventing Youth Suicide: Tips for Parents and Educators, National Association of School Psychologists

SOME-129 • *same as HELP-213:* The School Counselor and Suicide Risk Assessment, American School Counselor Association

SOME-130 • How to Talk to a Friend About Mental Health or Drugs, Centers for Disease Control and Prevention

SOME-131 • Suicide Prevention Toolkit for Caregivers (PDF), U.S. Department of Veterans Affairs

Warning Signs of Suicide: https://www.nimh.nih.gov/health/publications/warning-signs-of-suicide

5 Action Steps to Help Someone Having Thoughts of Suicide: https://www.nimh.nih.gov/health/publications/5-action-steps-to-help-someone-having-thoughts-of-suicide

Frequently Asked Questions About Suicide: https://www.nimh.nih.gov/health/publications/suicide-faq

Suicide Prevention: https://www.samhsa.gov/mental-health/suicidal-behavior/prevention

988 Frequently Asked Questions: https://www.samhsa.gov/mental-health/988/faqs

Help Someone Else: https://988lifeline.org/help-someone-else/

Safety and support on social media: https://988lifeline.org/help-someone-else/support-on-social-media/

What should I do if my child or teen is thinking about suicide?: https://www.healthychildren.org/English/tips-tools/ask-the-pediatrician/Pages/what-should-i-do-if-my-child-is-thinking-about-suicide.aspx

Suicide in Children and Teens: https://www.aacap.org/AACAP/Families_and_Youth/Facts_for_Families/FFF-Guide/Teen-Suicide-010.aspx

Preventing Youth Suicide: Tips for Parents and Educators: https://www.nasponline.org/resources-and-publications/resources-and-podcasts/school-safety-and-crisis/mental-health-resources/preventing-youth-suicide/preventing-youth-suicide-tips-for-parents-and-educators

The School Counselor and Suicide Risk Assessment: https://www.schoolcounselor.org/Standards-Positions/Position-Statements/ASCA-Position-Statements/The-School-Counselor-and-Suicide-Risk-Assessment

How to Talk to a Friend About Mental Health or Drugs: https://www.cdc.gov/free-mind/teens/talk-to-a-friend.html

Suicide Prevention Toolkit for Caregivers (PDF): https://www.caregiver.va.gov/pdfs/va-suicide-prevention-toolkit-for-caregivers.PDF

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Page: For Teens

Address once published: https://stillwatersps.github.io/suicideprevention/teens/. Numbers start with TEEN.

TEEN-1 For teens

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

TEEN-2 **If you're a teen and things feel hard**

TEEN-3 You can reach out for support yourself, any time, even if there is no adult you can tell.

TEEN-4 Here's how, and what stays private.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

TEEN–5 **Right now** *same as HELP-5*

- TEEN–6 • *same as HELP-6:* Reach out to 988 with the call, text, or chat buttons above.
- TEEN–7 • *same as HELP-7:* You don't need to be sure it's an emergency.
- TEEN–8 • *same as HELP-8:* It's free and confidential.
- TEEN–9 • *same as HELP-9:* In immediate danger?
- TEEN–10 • *same as HELP-10:* Call 911.
- TEEN–11 • *same as HELP-11:* If one way doesn't connect, try another: call, text, or chat.
- TEEN–12 • *same as HELP-12:* The rest of this page can wait.
- TEEN–13 • *same as HELP-13:* Open a section below when you're ready.

TEEN–14 *same as HELP-14:* Draft for review.

TEEN–15 A suicide prevention professional has not reviewed this page yet, and its founders are still deciding which ages the site is designed for.

TEEN–16 *same as HELP-16:* An AI assistant drafted it and compared the phone numbers and links with each provider's website on November 6, 2026.

TEEN–17 *same as SHARED-16:* No person has confirmed them or tried them on a phone.

Call 911: `tel:911`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

TEEN–18 **You can reach out yourself**

TEEN–19 *same as HELP-160:* You can call or text 988 yourself, even if there is no adult you can tell.

TEEN–20 *same as HELP-161:* Its rules don't set a minimum age, and you don't have to give your name or age to get support.

TEEN–21 *same as HELP-162:* It can help to save 988 in your phone now, in case you need it later.

TEEN–22 **The Trevor Project** *same as HELP-102*

TEEN–23 *same as HELP-103:* For LGBTQ+ young people.

TEEN–24 *same as HELP-104:* Its terms describe the phone lifeline as for people under age 25.

TEEN–25 *same as HELP-105:* Trained counselors by phone, text, or chat, free and confidential, 24/7.

TEEN–26 *same as HELP-106:* Message and data rates may apply to texts.

TEEN–27 Call 1-866-488-7386 | Text START to 678-678

TEEN–28 **LGBTQ+ youth, through 988** *same as HELP-98*

TEEN–29 *same as HELP-99:* The 988 Lifeline says its specialized service for LGBTQ+ youth has been restored.

TEEN–30 *same as HELP-100:* It is separate from The Trevor Project.

TEEN–31 *same as HELP-101:* Call 988, then press 3 | Text PRIDE to 988

TEEN–32 **Crisis Text Line** *same as HELP-109*

TEEN–33 *same as HELP-110:* Trained volunteer crisis counselors by text, free and confidential, 24/7, anywhere in the United States.

TEEN–34 *same as HELP-111:* Messages are free on AT&T, T-Mobile, Sprint, and Verizon.

TEEN–35 *same as HELP-112:* On other carriers, standard rates apply and 741741 can appear on the bill.

TEEN–36 *same as HELP-113:* It says many T-Mobile prepaid plans don't allow numbers like 741741, so if a text doesn't go through, try another way.

TEEN–37 Text HOME to 741741

TEEN–38 More options, including Spanish, ASL, and other languages, are on Get Help.

Call 1-866-488-7386: `tel:+18664887386`
Text START to 678-678: `sms:678678`
Call 988, then press 3: `tel:988`
Text PRIDE to 988: `sms:988`
Text HOME to 741741: `sms:741741`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

TEEN–39 **Telling an adult you trust**

TEEN–40 *same as HELP-165:* It can also help to tell an adult you trust, such as a parent or guardian, a relative, a teacher, a coach, a spiritual leader, or a doctor.

TEEN–41 *same as HELP-166:* It's okay to ask what they can keep private first.

TEEN–42 *same as HELP-167:* If you tell a school counselor you are thinking about suicide, expect them to tell a parent or guardian.

TEEN–43 *same as HELP-168:* Their national association treats this as required, even for students who are 18.

TEEN–44 *same as HELP-169:* If a parent's abuse or neglect is the reason, the counselor contacts child protective services instead.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

TEEN–45 **What stays private** *same as HELP-51*

TEEN–46 *same as HELP-170:* Crisis lines keep conversations confidential, with limits.

TEEN–47 *same as HELP-171:* The 988 Lifeline says it may share information with 911, police, or a mobile crisis team, without your permission, in some situations.

TEEN–48 *same as HELP-172:* For example, if someone is in danger of being hurt very soon, or if a young person is being abused or neglected under state law.

TEEN–49 *same as HELP-173:* The Trevor Project and Crisis Text Line have similar limits.

TEEN–50 *same as HELP-174:* The Trevor Project says it won't tell parents or guardians unless the law or a subpoena requires it.

TEEN–51 *same as HELP-175:* A small number of crisis centers bill insurance and may ask for your details.

TEEN–52 *same as HELP-176:* You can say no and still get support.

TEEN–53 *same as HELP-177:* If you give them and a claim is filed, a parent or guardian could get an insurance statement.

TEEN–54 *same as HELP-60:* On a shared phone or computer, visits to this site can show up in browser history, and calls or texts can show up in phone logs.

TEEN–55 The Quick exit button leaves this site fast, but it can't clear your history.

TEEN–56 See what it can and can't hide.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

TEEN-57 **If you're worried about a friend**

TEEN-58 *same as SOME-93:* You don't need to have the answers.

TEEN-59 *same as SOME-94:* Start with a caring question and listen.

TEEN-60 *same as SOME-95:* If you're worried about suicide, it's okay to ask directly: "Are you thinking about suicide?

TEEN-61 *same as SOME-96:* Encourage your friend to talk to a trusted adult, such as a parent, coach, teacher, or school counselor, and talk to one yourself.

TEEN-62 *same as SOME-97:* If your friend talks about suicide or about hurting themselves, don't keep it a secret, even if they ask you to.

TEEN-63 *same as SOME-98:* Tell a trusted adult, or call or text 988 to ask what to do.

TEEN-64 *same as SOME-99:* You can contact 988 at any time, even if it's about a friend.

TEEN-65 *same as SOME-100:* If they say they are going to end their life, call 911 if it's an emergency.

TEEN-66 *same as SOME-101:* If you're with them and it is safe, stay with them.

TEEN-67 *same as SOME-102:* If they told you by text or online, keep talking if you can while you get an adult, or call or text 988.

TEEN-68 *same as SOME-103:* You don't have to handle this by yourself.

TEEN-69 More on this is on Help Someone.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

TEEN-70 **More on this site** *same as HELP-178*

TEEN-71 Not sure what you're feeling? | After a suicide loss

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

TEEN-72 **Sources** *same as HELP-180*

TEEN-73 *same as HELP-181:* Checked November 6, 2026.

TEEN-74 *same as HELP-182:* An AI assistant drafted these summaries for Stillwater, and no qualified person has reviewed them yet.

TEEN-75 *same as HELP-183:* The organizations listed here have not reviewed or endorsed them.

TEEN-76 **Show sources (12)**

- TEEN-77 *same as HELP-185:* What to Expect, 988 Suicide & Crisis Lifeline
- TEEN-78 *same as HELP-186:* Get Help, 988 Suicide & Crisis Lifeline
- TEEN-79 *same as HELP-191:* Confidentiality, 988 Suicide & Crisis Lifeline
- TEEN-80 *same as HELP-192:* Terms of Service, 988 Suicide & Crisis Lifeline
- TEEN-81 *same as HELP-200:* For Young People Looking for Help, SAMHSA
- TEEN-82 *same as HELP-202:* Get Help, The Trevor Project

TEEN–83 • *same as HELP-203:* Terms of Service, The Trevor Project

TEEN–84 • *same as HELP-204:* FAQ, Crisis Text Line

TEEN–85 • *same as HELP-205:* Privacy Policy, Crisis Text Line

TEEN–86 • *same as HELP-209:* Child and Adolescent Mental Health, National Institute of Mental Health

TEEN–87 • *same as HELP-213:* The School Counselor and Suicide Risk Assessment, American School Counselor Association

TEEN–88 • *same as SOME-130:* How to Talk to a Friend About Mental Health or Drugs, Centers for Disease Control and Prevention

What to Expect: https://988lifeline.org/get-help/what-to-expect/

Get Help: https://988lifeline.org/get-help/

Confidentiality: https://988lifeline.org/confidentiality/

Terms of Service: https://988lifeline.org/terms-of-service/

For Young People Looking for Help: https://www.samhsa.gov/mental-health/what-is-mental-health/how-to-talk/young-people

Get Help: https://www.thetrevorproject.org/get-help/

Terms of Service: https://www.thetrevorproject.org/terms-of-service/

FAQ: https://www.crisistextline.org/about-us/faq/

Privacy Policy: https://www.crisistextline.org/privacy-policy/

Child and Adolescent Mental Health: https://www.nimh.nih.gov/health/topics/child-and-adolescent-mental-health

The School Counselor and Suicide Risk Assessment: https://www.schoolcounselor.org/Standards-Positions/Position-Statements/ASCA-Position-Statements/The-School-Counselor-and-Suicide-Risk-Assessment

How to Talk to a Friend About Mental Health or Drugs: https://www.cdc.gov/free-mind/teens/talk-to-a-friend.html

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Page: Not sure what you're feeling?

Address once published: https://stillwatersps.github.io/suicideprevention/not-sure/. Numbers start with SURE.

SURE–1 A gentle place to start

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SURE–2 **Not sure what you're feeling? Start here.**

SURE–3 You don't need to have the right words.

SURE–4 Take a moment to notice what is on your mind.

SURE–5 You can explore your feelings at your own pace, without pressure or judgment.

SURE–6 If you need support, there are people ready to listen.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SURE–7 **If you're thinking about suicide**

SURE–8 Even if you aren't sure, you can call 988 or text 988 now.

SURE–9 You don't need to be in an emergency or know what to say.

SURE–10 SAMHSA says there is no reason too big or too small to reach out.

SURE–11 *same as SHARED-3:* In immediate danger, call 911.

SURE–12 *same as HELP-14:* Draft for review.

SURE–13 *same as HELP-15:* A suicide prevention professional has not reviewed this page yet.

SURE–14 An AI assistant drafted it from a founder's brief and the sources listed below.

SURE–15 It is not a test, a diagnosis, or a replacement for care.

SURE–16 *same as SHARED-17:* See the review status of each page.

call 988: `tel:988`
text 988: `sms:988`
call 911: `tel:911`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SURE–17 **Before you start**

- SURE–18 • This isn't a test.
- SURE–19 • There are no scores and no right answers.
- SURE–20 • There is nothing to type.
- SURE–21 • What you open here isn't saved or sent.
- SURE–22 • You can skip any part, stop at any time, or go straight to talking with someone.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SURE–23 **What feeling is strongest right now?**

SURE–24 Open whichever one fits, more than one, or none at all.

SURE–25 **Overwhelmed**

SURE–26 Everyone feels stress from time to time, and the National Institute of Mental Health (NIMH) says it can sometimes feel overwhelming.

SURE–27 You don't have to work it all out right now.

SURE–28 NIMH suggests talking to people you trust about how you are feeling.

SURE–29 If you can't manage the stress, or it won't let up, a health care provider can help too.

SURE–30 **Sad**

SURE–31 NIMH says everyone feels sad or low sometimes, and these feelings usually pass.

SURE–32 If a low mood doesn't go away, or you think it could be depression, NIMH suggests talking to a health care provider.

SURE–33 This page can't tell you whether it's depression.

SURE-34 A provider can.

SURE-35 **Anxious**

SURE-36 NIMH says occasional anxiety is a normal part of life.

SURE-37 Many people worry about things like health, money, school, work, or family.

SURE-38 If it doesn't go away and starts to get in the way of your daily activities, NIMH suggests talking to a health care provider.

SURE-39 **Numb or empty**

SURE-40 Struggling doesn't always feel like sadness.

SURE-41 NIMH lists a lasting "empty" mood among the signs of depression, and says some older adults notice a lack of emotions instead of a low mood.

SURE-42 If you feel empty or hopeless, or feel you have no reason to live, please reach out now.

SURE-43 NIMH lists these feelings among the warning signs of suicide.

SURE-44 You can call or text 988 any time.

SURE-45 **Unsure, or something else**

SURE-46 That's okay.

SURE-47 You don't need a name for what you feel to talk with someone you trust or to contact 988.

SURE-48 *same as SURE-10:* SAMHSA says there is no reason too big or too small to reach out.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SURE-49 **What would you like to do next?**

- SURE-50 Speak with someone.
- SURE-51 Call 988, text 988, or reach out to someone you trust.
- SURE-52 See your options.
- SURE-53 Get Help lists other services and what to expect from them.
- SURE-54 If you've lost someone to suicide, After a loss may be a better place to start.
- SURE-55 Take a break.
- SURE-56 You can stop here and come back any time.
- SURE-57 This page keeps no record of what you open.
- SURE-58 Your browser may remember the visit, and the web host logs visitors' IP addresses.
- SURE-59 What this site can't hide.

Call 988: `tel:988`
text 988: `sms:988`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SURE-60 ## When to talk to a professional

SURE-61 NIMH suggests seeking professional help if you have severe or distressing symptoms that have lasted 2 weeks or more.

SURE-62 A primary care provider can refer you to a mental health professional.

SURE-63 NIMH has tips for that conversation, and SAMHSA's FindTreatment.gov can help you search for care near you.

SURE-64 You don't have to wait 2 weeks to reach out.

SURE-65 You can call or text 988 at any point, and call 911 in an emergency.

tips for that conversation: https://www.nimh.nih.gov/health/publications/tips-for-talking-with-your-health-care-provider

FindTreatment.gov: https://findtreatment.gov/

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

SURE-66 ## Sources *same as HELP-180*

SURE-67 *same as HELP-181:* Checked November 6, 2026.

SURE-68 *same as HELP-182:* An AI assistant drafted these summaries for Stillwater, and no qualified person has reviewed them yet.

SURE-69 *same as HELP-183:* The organizations listed here have not reviewed or endorsed them.

SURE-70 ## Show sources (8)

SURE-71 • *same as HELP-197:* 988 Frequently Asked Questions, Substance Abuse and Mental Health Services Administration (SAMHSA)

SURE-72 • I'm So Stressed Out!

SURE-73 • Fact Sheet, National Institute of Mental Health (NIMH)

SURE-74 • Depression, NIMH

SURE-75 • Generalized Anxiety Disorder: What You Need to Know, NIMH

SURE-76 • Caring for Your Mental Health, NIMH

SURE-77 • Frequently Asked Questions About Suicide, NIMH

SURE-78 • Tips for Talking With a Health Care Provider, NIMH

SURE-79 • *same as HELP-199:* FindTreatment.gov, SAMHSA

988 Frequently Asked Questions: https://www.samhsa.gov/mental-health/988/faqs

I'm So Stressed Out! Fact Sheet: https://www.nimh.nih.gov/health/publications/so-stressed-out-fact-sheet

Depression: https://www.nimh.nih.gov/health/publications/depression

Generalized Anxiety Disorder: What You Need to Know: https://www.nimh.nih.gov/health/publications/generalized-anxiety-disorder-gad

Caring for Your Mental Health: https://www.nimh.nih.gov/health/topics/caring-for-your-mental-health

Frequently Asked Questions About Suicide: https://www.nimh.nih.gov/health/publications/suicide-faq

Tips for Talking With a Health Care Provider: https://www.nimh.nih.gov/health/publications/tips-for-talking-with-your-health-care-provider

FindTreatment.gov: https://findtreatment.gov/

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Page: After a loss

Address once published: https://stillwatersps.github.io/suicideprevention/after-a-loss/. Numbers start with LOSS.

LOSS–1 After a suicide loss

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

LOSS–2 ## If you've lost someone to suicide

LOSS–3 There is no right way to grieve, and you don't have to go through this alone.

LOSS–4 Here is what groups that support people after a suicide loss say, and where to find people who understand.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

LOSS–5 **Right now** *same as HELP-5*

- LOSS–6 If this feels like too much, you can call or text 988 any time, even just to talk.
- LOSS–7 Thoughts of suicide can come with this kind of grief.
- LOSS–8 If they do, please reach out.
- LOSS–9 *same as SHARED-3:* In immediate danger, call 911.
- LOSS–10 *same as HELP-12:* The rest of this page can wait.
- LOSS–11 Read it when you're ready, a little at a time.

LOSS–12 *same as HELP-14:* Draft for review.

LOSS–13 A suicide prevention professional has not reviewed this page yet, and neither has anyone who has lost someone to suicide.

LOSS–14 An AI assistant drafted it from the sources listed below.

LOSS–15 *same as SHARED-17:* See the review status of each page.

call 911: `tel:911`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

LOSS–16 ### It's not your fault

LOSS–17 Many people keep asking why, and whether they could have done something.

LOSS–18 The CDC says suicide doesn't have just one cause.

LOSS–19 The Tragedy Assistance Program for Survivors (TAPS), which supports people after a military suicide loss, says it plainly: it's not your fault.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

LOSS–20 ## What you may feel

LOSS–21 The American Foundation for Suicide Prevention (AFSP) says everyone experiences a suicide loss in their own way, so no list will fit exactly.

LOSS–22 It says many people feel some mix of these:

LOSS–23 • Shock and disbelief

LOSS–24 • Grief, and yearning for the person

LOSS–25 • Anger

LOSS–26 • Guilt and blame

LOSS–27 • Confusion

LOSS–28 • Relief

LOSS–29 • Feeling rejected, abandoned, or betrayed

LOSS–30 • Shame and isolation

LOSS–31 • Depression and sadness

LOSS–32 • Thoughts of suicide

LOSS–33 **Show moreShow less** *same as HELP-22*

LOSS–34 AFSP says that if you saw what happened or found the person, you are likely to have trauma symptoms as well as grief.

LOSS–35 This can also happen if you weren't there.

LOSS–36 It may be hard to concentrate because of images that stay with you.

LOSS–37 You may feel anxious or confused, or have physical symptoms such as stomach problems or trouble sleeping.

LOSS–38 AFSP says these are normal responses to trauma and will likely ease over weeks and months.

LOSS–39 If they don't, it suggests seeing a mental health professional with experience in trauma or loss.

LOSS–40 Birthdays, holidays, and other meaningful dates can be hard.

LOSS–41 AFSP says that waiting for the day can be harder than the day itself.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

LOSS–42 ## Taking care of yourself

LOSS–43 Suicide Awareness Voices of Education (SAVE) says healing from a suicide loss is deeply personal, and no one approach works best for everyone.

LOSS–44 AFSP says you shouldn't have to cope with your loss alone.

LOSS–45 It suggests being kind and patient with yourself, and being open to letting people help.

LOSS–46 TAPS says the intense pain will not always feel the way it does right now, and that it can ease and soften with support and with time.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

LOSS–47 **Where to find support**

LOSS–48 These are separate organizations with their own rules.

LOSS–49 Stillwater does not run them, and has not checked each group they list.

LOSS–50 **Talk with someone who has been there**

LOSS–51 AFSP's Healing Conversations is a one-time visit, by phone, online, or in person, with a trained volunteer who has also lost someone to suicide.

LOSS–52 You ask for one through a short form on AFSP's website, so AFSP sees what you share there.

LOSS–53 AFSP Healing Conversations

LOSS–54 **Support groups**

LOSS–55 AFSP lists suicide loss support groups in the U.S. and elsewhere.

LOSS–56 It says it doesn't run, recommend, endorse, or fund the groups it lists, and that groups are in English unless stated.

LOSS–57 SAVE says it doesn't verify groups in outside lists.

LOSS–58 It's okay to ask how a group works before you join.

LOSS–59 AFSP support group list | SAVE support groups

LOSS–60 **A therapist or counselor**

LOSS–61 AFSP keeps a list of clinicians who completed its former training or say they have relevant experience with suicide loss.

LOSS–62 It says being listed is not an endorsement or referral.

LOSS–63 You can also ask a health care provider for a referral, or search for care near you.

LOSS–64 AFSP clinician list | FindTreatment.gov

LOSS–65 **After a military loss**

LOSS–66 TAPS supports anyone who has lost a family member, loved one, colleague, friend, or battle buddy to suicide, during or after their service.

LOSS–67 Its services include peer mentors who have also lost someone to suicide.

LOSS–68 TAPS suicide loss support

LOSS–69 **After the death of a child, grandchild, or sibling**

LOSS–70 The Compassionate Friends supports families after the death of a child, with local chapters and online groups.

LOSS–71 The Compassionate Friends

LOSS-72 **En español** *same as HELP-76*

LOSS-73 AFSP tiene una guía sobre cómo sobrellevar una pérdida por suicidio.

LOSS-74 La Línea 988 también ofrece apoyo en español: marca 988 y luego presiona 2, o envía AYUDA al 988.

LOSS-75 Sobreviviendo una pérdida por suicidio | Esta página en español

LOSS-76 AFSP also has a longer guide, Surviving a Suicide Loss, and answers to practical questions about the first few days.

AFSP Healing Conversations: `https://afsp.org/healing-conversations/`
AFSP support group list: `https://afsp.org/find-a-support-group/`
SAVE support groups: `https://www.save.org/programs/suicide-loss-support/find-a-support-group/`
AFSP clinician list: `https://afsp.org/suicide-bereavement-trained-clinicians/`
FindTreatment.gov: `https://findtreatment.gov/`
TAPS suicide loss support: `https://www.taps.org/suicide`
The Compassionate Friends: `https://www.compassionatefriends.org/`
Sobreviviendo una pérdida por suicidio: `https://aws-fetch.s3.amazonaws.com/flipbooks/sobreviviendoUnaPerdidaPorSuicidio/index.html`
Surviving a Suicide Loss: `https://aws-fetch.s3.amazonaws.com/flipbooks/survivingasuicideloss/index.html`
practical questions about the first few days: `https://afsp.org/practical-information-for-immediately-after-a-loss/`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

LOSS-77 **Talking to others about what happened**

LOSS-78 You may not want to tell people how the person died, and that's your choice.

LOSS-79 AFSP says that, in the long run, most people are glad they were honest, because it gives friends and family the chance to support them.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

LOSS-80 **Talking with children and teens**

LOSS-81 AFSP says it is up to parents and guardians whether to tell a child the truth, and points out that children may overhear adults talking.

LOSS-82 When you do talk with a child or teen, it suggests that you:

- LOSS-83 Give truthful information, encourage questions, and offer loving reassurance.
- LOSS-84 Reassure them that they are not responsible, and that nothing they said or did caused it.
- LOSS-85 Be ready to talk about it many times, in the first weeks and later in their life.
- LOSS-86 Consider a children's grief support group if they are having a hard time.

LOSS-87 The Dougy Center's grief resources | Children, Teens and Suicide Loss (AFSP booklet)

LOSS-88 If you're a teen who has lost someone, The Dougy Center has resources for you.

LOSS-89 It can also help to tell an adult you trust.

LOSS-90 If things feel too hard, you can call or text 988 yourself.

LOSS-91 More for teens.

The Dougy Center's grief resources: https://www.dougy.org/grief-support-resources

Children, Teens and Suicide Loss (AFSP booklet): https://aws-fetch.s3.amazonaws.com/flipbooks/childrenteenssuicideloss/index.html?page=1

resources for you: https://www.dougy.org/grief-support-resources/teens

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

LOSS-92 ## Schools, workplaces, and communities

LOSS-93 Support after a suicide is sometimes called postvention.

LOSS-94 These guides are for people responding to a death in a school, workplace, or community:

- LOSS-95 After a Suicide: A Toolkit for Schools, from AFSP and the Suicide Prevention Resource Center
- LOSS-96 After a Suicide: Postvention Toolkit for Workplaces, listed by AFSP
- LOSS-97 Responding to Grief, Trauma, and Distress After a Suicide: U.S. National Guidelines, from the National Action Alliance for Suicide Prevention

After a Suicide: A Toolkit for Schools: https://sprc.org/resources/after-suicide-toolkit-schools

After a Suicide: Postvention Toolkit for Workplaces: https://aws-fetch.s3.amazonaws.com/flipbooks/afterASuicidePostventionToolkitForWorkplaces/index.html

Responding to Grief, Trauma, and Distress After a Suicide: U.S. National Guidelines: https://theactionalliance.org/resource/responding-grief-trauma-and-distress-after-suicide-us-national-guidelines

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

LOSS-98 ## Sources *same as HELP-180*

LOSS-99 *same as HELP-181:* Checked November 6, 2026.

LOSS-100 *same as HELP-182:* An AI assistant drafted these summaries for Stillwater, and no qualified person has reviewed them yet.

LOSS-101 *same as HELP-183:* The organizations listed here have not reviewed or endorsed them.

LOSS-102 ### Show sources (17)

- LOSS-103 I've lost someone, American Foundation for Suicide Prevention (AFSP)
- LOSS-104 Taking care of yourself, AFSP
- LOSS-105 Talking to others about what happened, AFSP
- LOSS-106 Healing Conversations, AFSP
- LOSS-107 Find a support group, AFSP
- LOSS-108 Suicide bereavement trained clinicians, AFSP
- LOSS-109 Suicide loss resources, AFSP
- LOSS-110 What's next: Grief, healing, and life after a suicide loss, Suicide Awareness Voices of Education (SAVE)
- LOSS-111 Find a support group, SAVE

LOSS–112 • About Suicide Prevention, Centers for Disease Control and Prevention (CDC)

LOSS–113 • *same as SOME-123:* 988 Frequently Asked Questions, SAMHSA

LOSS–114 • Suicide Loss Support, Tragedy Assistance Program for Survivors (TAPS)

LOSS–115 • *same as LOSS-71:* The Compassionate Friends

LOSS–116 • Grief Support and Resources, The Dougy Center

LOSS–117 • *same as HELP-199:* FindTreatment.gov, SAMHSA

LOSS–118 • After a Suicide: A Toolkit for Schools, AFSP and the Suicide Prevention Resource Center

LOSS–119 • Responding to Grief, Trauma, and Distress After a Suicide: U.S. National Guidelines, National Action Alliance for Suicide Prevention

I've lost someone: https://afsp.org/ive-lost-someone/
Taking care of yourself: https://afsp.org/taking-care-of-yourself/
Talking to others about what happened: https://afsp.org/talking-to-others-about-what-happened/
Healing Conversations: https://afsp.org/healing-conversations/
Find a support group: https://afsp.org/find-a-support-group/
Suicide bereavement trained clinicians: https://afsp.org/suicide-bereavement-trained-clinicians/
Suicide loss resources: https://afsp.org/suicide-loss-resources/
What's next: Grief, healing, and life after a suicide loss: https://www.save.org/programs/suicide-loss-support/after-a-suicide-loss/
Find a support group: https://www.save.org/programs/suicide-loss-support/find-a-support-group/
About Suicide Prevention: https://www.cdc.gov/suicide/about/index.html
988 Frequently Asked Questions: https://www.samhsa.gov/mental-health/988/faqs
Suicide Loss Support: https://www.taps.org/suicide
The Compassionate Friends: https://www.compassionatefriends.org/
Grief Support and Resources: https://www.dougy.org/grief-support-resources
FindTreatment.gov: https://findtreatment.gov/
After a Suicide: A Toolkit for Schools: https://sprc.org/resources/after-suicide-toolkit-schools
Responding to Grief, Trauma, and Distress After a Suicide: U.S. National Guidelines: https://theactionalliance.org/resource/responding-grief-trauma-and-distress-after-suicide-us-national-guidelines

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Page: Research

Address once published: https://stillwatersps.github.io/suicideprevention/research/. Numbers start with RES.

RES–1 Learn more

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

RES–2 ## What the research shows

RES–3 Studies can show what tends to help, and where the evidence is still thin.

RES–4 Thin evidence usually means there aren't enough strong studies yet, not that help doesn't work.

RES–5 *same as HELP-14:* Draft for review.

RES–6 *same as HELP-15:* A suicide prevention professional has not reviewed this page yet.

RES–7 An AI assistant drafted these plain-language summaries for Stillwater, and every number should be checked against its source before people rely on it.

RES–8 Links were checked on November 6, 2026.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

RES–9 **Myths and facts**

RES–10 Some common worries stop people from asking for help, or from helping someone else.

RES–11 Here is what the sources on this site say.

RES–12 **Asking someone about suicide does not put the idea in their head**

RES–13 It's a common worry.

RES–14 *same as SOME-40:* NIMH says studies show that asking does not increase suicidal thoughts or behavior.

RES–15 What the research shows about asking

RES–16 **Talk of suicide should always be taken seriously**

RES–17 Some people think it's just a bid for attention.

RES–18 NIMH advises taking all talk of suicide seriously.

RES–19 *same as SOME-58:* What helps in the conversation

RES–20 **You don't need to be in an emergency to contact 988**

RES–21 *same as HELP-20:* The 988 Lifeline says anyone who is struggling can reach out, whether you are depressed, need to talk, or are thinking about suicide.

RES–22 You can also contact it if you're worried about someone.

RES–23 *same as HELP-18:* What happens when you reach out to 988

RES–24 **Most 988 calls don't involve emergency services**

RES–25 Some people worry that reaching out means the police will come.

RES–26 The 988 Lifeline reports that fewer than 2 in 100 calls involve emergency services, and that more than half of those happen with the caller's consent.

RES–27 This is not a guarantee.

RES–28 *same as HELP-64:* When emergency services get involved

RES–29 **Effective help exists, and recovery is possible**

RES–30 NIMH says effective treatments are available for people at risk of suicide.

RES–31 *same as HELP-126:* The American Foundation for Suicide Prevention says recovery is possible with evidence-based treatment for depression and other mental health conditions.

RES–32 What the research shows about treatment

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

RES–33 **Crisis lines such as 988**

RES–34 **Many callers say the call helped**

RES–35 Researchers interviewed 437 adults who had called the Lifeline while having suicidal thoughts.

RES–36 About 7 in 10 said the call helped a lot, and about 1 in 4 said it helped a little.

RES–37 What it can't show: These are callers' own reports, with no comparison group, from before the 988 number launched.

RES–38 Counselors chose which callers to ask, callers who needed emergency services were not asked, and only people who agreed and could be reached were interviewed.

RES–39 Some people had suicidal thoughts again later.

RES–40 If that happens, you can call, text, or chat again.

RES–41 Gould and colleagues, Suicide and Life-Threatening Behavior, 2025

RES–42 **Studies of crisis lines are still limited**

RES–43 A review of 33 studies found only low-strength evidence that crisis lines work.

RES–44 There was some support for people feeling better by the end of a contact, and little good evidence about what happens later.

RES–45 This means there are too few strong studies, not that crisis lines don't help.

RES–46 The authors still concluded that the evidence supports keeping and expanding crisis lines.

RES–47 What it can't show: The search ended in 2018, before 988, with little evidence on chat and none on text.

RES–48 Hoffberg and colleagues, Frontiers in Public Health, 2020

RES–49 **Early research since 988 launched**

RES–50 One short study found that suicide deaths among people aged 15 to 34 were lower than a statistical model predicted after 988 began in 2022.

RES–51 What it can't show: It cannot show that 988 caused this, because other things changed at the same time.

RES–52 Letters in JAMA questioned how deaths were counted and how differences between states were handled, and the authors replied.

RES–53 Patel and colleagues, JAMA, 2026

RES–54 Letter from Herring, JAMA, 2026

RES–55 Letter from Purtle and colleagues, JAMA, 2026

RES–56 Reply from the authors, JAMA, 2026

Gould and colleagues, Suicide and Life-Threatening Behavior, 2025: https://pubmed.ncbi.nlm.nih.gov/40405822/

Hoffberg and colleagues, Frontiers in Public Health, 2020: https://pubmed.ncbi.nlm.nih.gov/32010655/

Patel and colleagues, JAMA, 2026: https://pubmed.ncbi.nlm.nih.gov/42018312/

Letter from Herring, JAMA, 2026: https://pubmed.ncbi.nlm.nih.gov/42606846/

Letter from Purtle and colleagues, JAMA, 2026: https://pubmed.ncbi.nlm.nih.gov/42606880/

Reply from the authors, JAMA, 2026: https://pubmed.ncbi.nlm.nih.gov/42606879/

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

RES–57 **Asking someone about suicide**

RES–58 **Asking does not appear to raise suicidal thoughts**

RES–59 A review that pooled 8 randomized trials found no significant increase in suicidal thoughts or behavior, self-harm, or distress among people who were asked about these topics.

RES–60 A 2026 experiment with 700 U.S. teens also found that anonymous suicide questionnaires did not raise the desire to self-injure or to die right afterward.

RES–61 What it can't show: These studies used questionnaires, interviews, and screening, not conversations between friends or relatives.

RES–62 The teen study measured only immediate effects, and the review's authors asked for more high-quality trials.

RES–63 Polihronis and colleagues, Archives of Suicide Research, 2022

RES–64 McGuire and colleagues, Psychiatry Research, 2026

Polihronis and colleagues, Archives of Suicide Research, 2022: https://pubmed.ncbi.nlm.nih.gov/32715986/
McGuire and colleagues, Psychiatry Research, 2026: https://pubmed.ncbi.nlm.nih.gov/41747434/

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

RES–65 **Safety plans**

RES–66 **Safety planning is linked to less suicidal behavior in adults**

RES–67 A review of 6 studies in adults found that people who received a safety planning-type intervention had a lower risk of later suicidal behavior than people in comparison groups.

RES–68 It found no significant effect on suicidal thoughts.

RES–69 What it can't show: Four of the 6 studies were rated at high risk of bias, the pattern of results suggests some less favorable studies may be missing, and no children or teens were included.

RES–70 Some of these programs paired the plan with clinician contact and follow-up calls, so the results don't show what a plan made alone would do.

RES–71 Nuij and colleagues, British Journal of Psychiatry, 2021

RES–72 **For children and teens, the evidence is thinner**

RES–73 A review of brief programs built around a safety plan for children and teens found no clear benefit, based on 10 studies, 5 of which could be pooled.

RES–74 Many of these programs also included family sessions or follow-up calls.

RES–75 What it can't show: The evidence is thin, mostly small studies with moderate to high risk of bias.

RES–76 A benefit has not been shown, and these studies are too small to settle small effects in either direction.

RES–77 Albaum and colleagues, JAMA Pediatrics, 2025, corrected 2026

Nuij and colleagues, British Journal of Psychiatry, 2021: https://pubmed.ncbi.nlm.nih.gov/35048835/

Albaum and colleagues, JAMA Pediatrics, 2025, corrected 2026: https://jamanetwork.com/journals/jamapediatrics/fullarticle/2834286

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

RES-78 ## Staying in touch after a crisis

RES-79 ### Follow-up may help after an attempt, but results are mixed

RES-80 A 2026 review of randomized trials in adults who had attempted suicide found fewer repeat attempts after a mix of brief therapy and follow-up contact.

RES-81 It could not test follow-up contact alone, and it found no clear effect on self-harm or on connecting people to care.

RES-82 A separate 2026 trial with 709 American Indian and Alaska Native adults at high suicide risk added culturally adapted caring messages to help connecting with local resources.

RES-83 People in both groups had fewer suicidal thoughts and attempts over the year, and the messages added no clear benefit.

RES-84 The study can't say why both groups improved.

RES-85 What it can't show: The review covered programs in treatment and emergency settings.

RES-86 The trial ran in tribal communities, and about a third of participants had no 12-month data.

RES-87 Neither result may apply to other groups or settings.

RES-88 Stillwater does not offer follow-up contact.

RES-89 Homan and colleagues, eClinicalMedicine, 2026

RES-90 Comtois and colleagues, JAMA Network Open, 2026

Homan and colleagues, eClinicalMedicine, 2026: https://pubmed.ncbi.nlm.nih.gov/41859679/
Comtois and colleagues, JAMA Network Open, 2026: https://jamanetwork.com/journals/jamanetworkopen/fullarticle/2856150

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

RES-91 ## Safer surroundings

RES-92 ### Making dangerous items and places harder to reach has support

RES-93 An overview of 20 research reviews found good support for making dangerous items and places harder to reach across whole populations, with several approaches that appear to work.

RES-94 What it can't show: These reviews studied laws, sales rules, and structural changes across whole populations, not steps a family takes at home.

RES-95 The strength of the evidence varies by approach, and most of it is observational.

RES-96 Twelve of the 20 reviews got the lowest quality rating, mostly for reporting gaps, and many draw on the same studies.

RES-97 Steeg and colleagues, BMJ Mental Health, 2025.

RES-98 Not linked here because the paper names methods.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

RES-99 **Treatment**

RES-100 **Some therapies have evidence, with limited certainty**

RES-101 A Cochrane review of 76 trials in adults who self-harm called the evidence for most psychosocial treatments uncertain.

RES-102 It found low-certainty evidence that cognitive behavioral therapy may mean fewer people repeat self-harm at longer follow-up.

RES-103 A review of treatments for young people also called the evidence uncertain overall, with moderate evidence that dialectical behavior therapy reduces suicidal thoughts.

RES-104 What it can't show: These reviews looked at different ages and outcomes, so they can't be compared directly.

RES-105 Another analysis of 26 trials in teens found that no one therapy worked better than the care the other group received, for repeat self-harm at 12 months.

RES-106 That doesn't mean care doesn't help.

RES-107 A health care provider can talk through what might fit you.

RES-108 Witt and colleagues, Cochrane Database of Systematic Reviews, 2021

RES-109 Sim and colleagues, JAMA Pediatrics, 2025

RES-110 Cottrell and colleagues, Health Technology Assessment, 2026

Witt and colleagues, Cochrane Database of Systematic Reviews, 2021: https://pmc.ncbi.nlm.nih.gov/articles/PMC8094743/
Sim and colleagues, JAMA Pediatrics, 2025: https://pubmed.ncbi.nlm.nih.gov/41021221/
Cottrell and colleagues, Health Technology Assessment, 2026: https://pubmed.ncbi.nlm.nih.gov/41556887/

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

RES-111 **How suicide is talked about**

RES-112 **News coverage can matter**

RES-113 A review found that suicides rose on average in the weeks after the media reported a celebrity's death by suicide.

RES-114 This is one reason safe messaging guidance exists.

RES-115 If a death in the news is weighing on you, you can call or text 988 any time.

RES-116 What it can't show: It found no clear rise after general news reporting on suicide, though only 5 studies looked at that.

RES-117 Results varied widely, and some studies showing no effect may be unpublished.

RES-118 This kind of study cannot prove cause, and it does not cover talking with someone you care about.

RES-119 Niederkrotenthaler and colleagues, BMJ, 2020.

RES-120 *same as RES-98:* Not linked here because the paper names methods.

RES-121 **Stories of coping may help a little**

RES-122 When researchers combined several controlled trials, stories of people recovering from a suicidal crisis slightly reduced suicidal thoughts in people already at some risk.

RES-123 What it can't show: The effect is small, and there was not enough evidence to say whether these stories change attitudes toward seeking help.

RES-124 Niederkrotenthaler and colleagues, The Lancet Public Health, 2022

Niederkrotenthaler and colleagues, The Lancet Public Health, 2022: https://pubmed.ncbi.nlm.nih.gov/35122759/

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

RES-125 **What this page can't tell you**

RES-126 Research describes groups of people.

RES-127 It can't predict what will happen for any one person.

RES-128 No study has tested Stillwater, and nothing here shows that this website prevents suicide.

RES-129 In one trial on a Dutch helpline's website, a short message did not increase use of the helpline link.

RES-130 People who saw it did rate themselves as more likely to contact the helpline later, but the study could not check whether they did (van der Burgt and colleagues, JMIR Mental Health, 2024).

RES-131 Evidence specific to minors is thin.

RES-132 Thin evidence usually means there aren't enough strong studies yet.

RES-133 It doesn't mean help doesn't work, or that nothing can help you.

RES-134 Most people who go through hard times find support, treatment, or other ways to cope.

RES-135 *same as SURE-44:* You can call or text 988 any time.

RES-136 *same as HELP-182:* An AI assistant drafted these summaries for Stillwater, and no qualified person has reviewed them yet.

RES-137 The authors and journals have not reviewed them either.

RES-138 *same as RES-8:* Links were checked on November 6, 2026.

van der Burgt and colleagues, JMIR Mental Health, 2024: https://pubmed.ncbi.nlm.nih.gov/39235321/

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Page: Wallet card and poster

Address once published: https://stillwatersps.github.io/suicideprevention/card/. Numbers start with CARD.

CARD-1 Print and share

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

CARD-2 ## Print a wallet card or poster

CARD-3 Keep 988 close, or hand cards out at a school, clinic, or community space.

CARD-4 A printed card stays readable with no signal, but calling or texting needs phone service.

CARD-5 *same as HELP-14:* Draft for review.

CARD-6 *same as HELP-15:* A suicide prevention professional has not reviewed this page yet.

CARD-7 Please don't print copies to hand out until it has been reviewed.

CARD-8 *same as HELP-16:* An AI assistant drafted it and compared the phone numbers and links with each provider's website on November 6, 2026.

CARD-9 *same as SHARED-16:* No person has confirmed them or tried them on a phone.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

CARD-10 ### Poster

CARD-11 *same as HOME-2:* You are not alone.

CARD-12 Call or text 988

CARD-13 988 Suicide & Crisis Lifeline.

CARD-14 *same as HELP-95:* Free, confidential, and available 24/7.

CARD-15 You can also chat at 988lifeline.org.

- CARD-16 En español: llama al 988 y marca 2, o envía AYUDA al 988.
- CARD-17 Veterans and service members: call 988, then press 1, or text 838255.
- CARD-18 LGBTQ+ young people: The Trevor Project, call 1-866-488-7386 or text START to 678-678.
- CARD-19 Deaf or hard of hearing: dial 988 from a videophone, or use a relay service.

CARD-20 *same as SHARED-3:* In immediate danger, call 911.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

CARD-21 ### Wallet cards, eight to a page

CARD-22 Cut along the dashed lines.

CARD-23 Need to talk?

CARD-24 *same as CARD-12:* Call or text 988

CARD-25 *same as CARD-13:* 988 Suicide & Crisis Lifeline.

CARD-26 Free, confidential, 24/7.

CARD-27 Chat: 988lifeline.org

CARD-28 *same as CARD-16:* En español: llama al 988 y marca 2, o envía AYUDA al 988.

CARD-29 Veterans: call 988, then press 1.

CARD-30 *same as SHARED-3:* In immediate danger, call 911.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Page: About and privacy

Address once published: https://stillwatersps.github.io/suicideprevention/about/. Numbers start with ABOUT.

ABOUT–1 About

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

ABOUT–2 ## About Stillwater and your privacy

ABOUT–3 What this site is, what it keeps, what it can't hide, and what has and hasn't been reviewed.

ABOUT–4 *same as HELP-14:* Draft for review.

ABOUT–5 A suicide prevention professional has not reviewed this site yet, and a lawyer has not read this page.

ABOUT–6 An AI assistant drafted it.

ABOUT–7 The behavior it describes was tested in a desktop browser, and hasn't been tested on phones yet.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

ABOUT–8 ### What Stillwater is

ABOUT–9 *same as SHARED-5:* Stillwater is an early project to help people in the United States find suicide prevention support.

ABOUT–10 It is a website with information and links.

ABOUT–11 Stillwater is not a crisis service or medical care.

ABOUT–12 Nobody at Stillwater watches this site or can see who is using it, and there is no way to send a message through it.

ABOUT–13 For help, use the call, text, and chat options at the top of each page.

ABOUT–14 They connect you to outside services, such as the 988 Suicide & Crisis Lifeline.

ABOUT–15 An AI assistant drafted the text on this site for the two people starting this project.

ABOUT–16 Stillwater has not been tested as a way to prevent suicide.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

ABOUT–17 ### What has been reviewed

ABOUT–18 No licensed clinician has reviewed this site yet, and no fluent reader has checked the Spanish.

ABOUT–19 When a review is finished, this list will show the reviewer's role and the date.

ABOUT-20 Until then, every page stays marked as a draft.

ABOUT-21 • HomeClinical review: PendingSpanish version, fluent reader: Pending

ABOUT-22 • Get HelpClinical review: PendingSpanish version, fluent reader: Pending

ABOUT-23 • Help SomeoneClinical review: PendingSpanish version, fluent reader: Pending

ABOUT-24 • For TeensClinical review: PendingSpanish version, fluent reader: Pending

ABOUT-25 • ResearchClinical review: PendingSpanish version, fluent reader: Pending

ABOUT-26 • Not sure what you're feeling?Clinical review: PendingSpanish version, fluent reader: Pending

ABOUT-27 • After a lossClinical review: PendingSpanish version, fluent reader: Pending

ABOUT-28 • Wallet card and posterClinical review: PendingSpanish version, fluent reader: Pending

ABOUT-29 • About and privacyClinical review: PendingSpanish version, fluent reader: Pending

ABOUT-30 Crisis phone numbers and links: an AI assistant compared them with each provider's website on November 6, 2026.

ABOUT-31 *same as SHARED-16:* No person has confirmed them or tried them on a phone.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

ABOUT-32 **Your privacy**

ABOUT-33 • What this site collects.

ABOUT-34 • It has no sign-ups, forms, analytics, ads, or cookies, and it doesn't load anything from other websites.

ABOUT-35 • The web host.

ABOUT-36 • This site is hosted on GitHub Pages.

ABOUT-37 • GitHub says it logs and stores visitors' IP addresses for security, whether or not they are signed in to GitHub.

ABOUT-38 • Other services.

ABOUT-39 • When you call, text, chat, or open a link to another website, that service's own rules apply.

ABOUT-40 • See what happens when you reach out to 988.

ABOUT-41 • On a shared phone or computer.

ABOUT-42 • Visits to this site can show up in browser history, and calls or texts can show up in phone logs.

GitHub says it logs and stores visitors' IP addresses: `https://docs.github.com/en/pages/getting-started-with-github-pages/what-is-github-pages#data-collection`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

ABOUT-43 **What Quick exit can and can't do**

ABOUT-44 The Quick exit button opens the weather.gov home page in the same tab.

ABOUT-45 It also replaces the page you were on in that tab's history, so Back won't return to that page.

ABOUT-46 It can't:

ABOUT-47 • Remove other Stillwater pages you opened earlier in that tab.

ABOUT-48 • Back can still reach them.

ABOUT-49 • Clear your browser's history, other tabs, or the saved copies described below.

ABOUT-50 • Remove a contact you saved, or calls and texts from your phone's logs.

ABOUT-51 If JavaScript is turned off, Quick exit is an ordinary link, so Back can return to the page you left.

ABOUT-52 With no connection, weather.gov can't load, and your browser shows its own error page instead.

ABOUT-53 A private or incognito window can keep pages out of this device's history.

ABOUT-54 To remove pages you've already visited, clear your history and website data in your browser's settings.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

ABOUT-55 **Saved copies and using this site offline**

ABOUT-56 On a secure connection, your browser may save a copy of this site's pages and files after your first visit, so the crisis numbers can still show with a weak signal or none.

ABOUT-57 • What works offline.

ABOUT-58 • After that first visit, the pages of this site can open with no connection, in English and Spanish.

ABOUT-59 • An address that wasn't saved shows the "page not found" page, which has the crisis numbers.

ABOUT-60 • What doesn't.

ABOUT-61 • Calling and texting need phone service.

ABOUT-62 • Chat and links to other websites need an internet connection.

ABOUT-63 • How updates arrive.

ABOUT-64 • With a connection, the site tries for the newest version first, which can take up to about 10 minutes to appear.

ABOUT-65 • A saved copy is used only when the newest version can't load, so it could be out of date.

ABOUT-66 • When your device reports no connection, the page says so.

ABOUT-67 • If a number doesn't work, try 911 or another option on this page.

ABOUT-68 • Don't rely on it alone.

ABOUT-69 • Browsers can delete saved copies, for example when a device is low on storage or you haven't used the site for a while.

ABOUT-70 • Keeping 988 in your contacts or on a wallet card doesn't depend on this.

ABOUT-71 • *same as ABOUT-32:* Your privacy.

ABOUT-72 • Saved copies stay on your device.

ABOUT-73 • Nothing is sent to Stillwater, but on a shared device they are a trace of your visit, and Quick exit can't remove them.

ABOUT-74 • Clearing website data in your browser's settings does.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

ABOUT-75 **Saving 988 to your contacts**

ABOUT-76 The "Save 988 to your contacts" button opens a small contact card file with the 988 Lifeline's name, number, and website.

ABOUT-77 It comes from this site, and nothing about you is sent to Stillwater.

ABOUT-78 You choose whether to save it.

ABOUT-79 On a shared phone, or if your contacts sync to another device, other people may see it.

ABOUT-80 *same as SHARED-29:* Save 988 to your contacts

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

ABOUT-81 **Sources** *same as HELP-180*

ABOUT-82 *same as HELP-181:* Checked November 6, 2026.

ABOUT-83 *same as HELP-182:* An AI assistant drafted these summaries for Stillwater, and no qualified person has reviewed them yet.

ABOUT-84 *same as HELP-183:* The organizations listed here have not reviewed or endorsed them.

ABOUT-85 **Show sources (4)**

ABOUT-86 • What is GitHub Pages?

ABOUT-87 • (Data collection), GitHub

ABOUT-88 • Service Worker API, MDN

ABOUT-89 • Updates to Storage Policy, WebKit

ABOUT-90 • *same as HELP-191:* Confidentiality, 988 Suicide & Crisis Lifeline

What is GitHub Pages? (Data collection): `https://docs.github.com/en/pages/getting-started-with-github-pages/what-is-github-pages#data-collection`

Service Worker API: `https://developer.mozilla.org/en-US/docs/Web/API/Service_Worker_API`

Updates to Storage Policy: `https://webkit.org/blog/14403/updates-to-storage-policy/`

Confidentiality: `https://988lifeline.org/confidentiality/`

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Page: Page not found

Address once published: shown for any address that does not exist. Numbers start with NOTFOUND.

NOTFOUND-1 Page not found

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

NOTFOUND-2 **We can't find that page**

NOTFOUND-3 The link may be out of date.

NOTFOUND–4 The options below still work, and you can reach the pages of this site from here.

NOTFOUND–5 With no connection, this page can also appear in place of a page that wasn't saved.

NOTFOUND–6 ¿No encuentras la página?

NOTFOUND–7 Para hablar en español, llama al 988 y marca 2, o envía AYUDA al 988.

NOTFOUND–8 *same as HELP-82:* Si hay peligro inmediato, llama al 911.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

NOTFOUND–9 **Pages on this site**

NOTFOUND–10 Home | Get help for yourself | Help someone else | Not sure what you're feeling? | After a suicide loss | For teens | What the research shows | Wallet card | About and privacy

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Part 3: Research brief

A founder supplied this brief. The website's draft text was written separately and did not come from it, so please read both. The unchanged PDF is in the project at `notes/attachments/research-and-website-copy-2026-11.pdf`.

BRIEF-1 **Suicide Prevention Research & Website Content**

BRIEF-2 Research reference and implementation brief | Prepared November 2026

BRIEF-3 Purpose: Educational background for a suicide-prevention website.

BRIEF-4 This material is not a diagnostic tool or substitute for professional mental health care.

BRIEF-5 Statistics are year-specific and should be rechecked before publication.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

BRIEF-6 **1. Understanding Suicide and Suicidal Behavior**

BRIEF-7 Definitions.

BRIEF-8 Suicide is a death resulting from intentional self-directed behavior.

BRIEF-9 Suicidal ideation means thoughts about suicide; a suicide attempt is nonfatal suicidal behavior.

BRIEF-10 Suicide prevention includes interventions that reduce risk and strengthen access to care.

BRIEF-11 Public health perspective.

BRIEF-12 Suicide is influenced by overlapping individual, interpersonal, community and societal factors.

BRIEF-13 No one diagnosis or experience explains every case.

BRIEF-14 Myths and accurate framing.

BRIEF-15 Depression is one risk factor, not a universal cause.

BRIEF-16 Asking someone directly and compassionately about suicide does not implant the idea.

BRIEF-17 Prevention and recovery are possible.

BRIEF-18 Use nonjudgmental language such as "died by suicide."

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

BRIEF-19 **2. Causes and Risk Factors**

BRIEF-20 Psychological factors.

BRIEF-21 Depression, anxiety, trauma-related symptoms, substance use disorders and other mental health difficulties may contribute to vulnerability.

BRIEF-22 A diagnosis alone does not predict an individual outcome.

BRIEF-23 Biological factors.

BRIEF-24 Research investigates genetic vulnerability, stress-response systems, sleep disturbance and emotional regulation.

BRIEF-25 None is a deterministic explanation.

BRIEF-26 Social and environmental factors.

BRIEF-27 Bullying, discrimination, interpersonal conflict, loss, economic instability, violence, social isolation and limited access to care can add stress.

BRIEF-28 Risk factors can accumulate or interact.

BRIEF-29 Protective factors.

BRIEF-30 Supportive relationships, belonging, accessible treatment, coping skills, stable environments and continuity of care can reduce risk.

BRIEF-31 Protective factors help but do not guarantee safety.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

BRIEF-32 **3. Warning Signs and Recognizing Distress**

BRIEF-33 Emotional and behavioral indicators.

BRIEF-34 Potential concerns include persistent hopelessness, feeling trapped, intense shame, withdrawal, significant changes in sleep or functioning, agitation, and direct statements about suicide.

BRIEF-35 Warning signs are not a diagnosis.

BRIEF-36 Responding.

BRIEF-37 Listen calmly and without judgment.

BRIEF-38 Ask clearly about suicide when concerned.

BRIEF-39 Do not dismiss statements as attention-seeking or promise secrecy where safety is at risk.

BRIEF-40 Involve qualified help and a trusted adult for young people.

BRIEF-41 Urgent support.

BRIEF-42 In the United States, call or text 988 for crisis support.

BRIEF-43 For immediate life-threatening danger, contact emergency services.

BRIEF-44 A website should make these pathways easy to find.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

BRIEF-45 **4. Effects on Families, Friends, Schools and Communities**

BRIEF-46 Family and peer impact.

BRIEF-47 Suicide bereavement may involve grief, anxiety, confusion, anger, stigma and unanswered questions.

BRIEF-48 Survivors are not responsible for another person's death.

BRIEF-49 Schools and communities.

BRIEF-50 Schools may need coordinated counseling, careful communication, support for staff and students, and referral pathways.

BRIEF-51 Postvention means organized support after a suicide.

BRIEF-52 Responsible reporting.

BRIEF-53 Avoid sensationalism and simplistic explanations.

BRIEF–54 Communicate the availability of support and recovery; avoid framing suicide as inevitable.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

BRIEF–55 **5. Evidence-Based Prevention**

BRIEF–56 Three levels.

BRIEF–57 Primary prevention reduces upstream risks through connectedness, education and supportive environments.

BRIEF–58 Secondary prevention recognizes emerging concerns and connects people to care.

BRIEF–59 Tertiary prevention emphasizes recovery, follow-up and continuity of treatment.

BRIEF–60 Community approaches.

BRIEF–61 Strengthen access to affordable mental healthcare, address bullying and violence, promote social support, reduce harmful barriers and create clear referral pathways.

BRIEF–62 WHO LIVE LIFE framework.

BRIEF–63 WHO recommends a coordinated public health approach including responsible media communication, adolescent life skills, early identification and support, and safer environments.

BRIEF–64 Evaluation.

BRIEF–65 Track access to support, user comprehension, referral completion, privacy and accessibility.

BRIEF–66 Website engagement alone is not evidence of suicide prevention efficacy.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

BRIEF–67 **6. Treatment and Recovery**

BRIEF–68 Professional care.

BRIEF–69 Treatment may include cognitive behavioral therapy, dialectical behavior therapy, family or group therapy, psychiatric evaluation, and management of underlying conditions.

BRIEF–70 Selection depends on individual needs and qualified clinical assessment.

BRIEF–71 Safety planning.

BRIEF–72 A collaborative safety plan may identify personal warning signs, coping approaches, trusted contacts, professional support and steps to maintain safety.

BRIEF–73 It is not a substitute for treatment.

BRIEF–74 Continuity.

BRIEF–75 Recovery is not necessarily linear.

BRIEF–76 Follow-up, supportive relationships and adjusted care plans can help people manage renewed distress.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

BRIEF-77 **7. Statistics and Research Interpretation**

BRIEF-78 Global context.

BRIEF-79 WHO has reported more than 720,000 suicide deaths worldwide annually based on 2021 estimates.

BRIEF-80 Figures change with updated data and methods; check the latest WHO fact sheet before publishing exact figures.

BRIEF-81 United States context.

BRIEF-82 CDC surveillance reports tens of thousands of suicide deaths annually, with substantially larger numbers reporting suicidal thoughts.

BRIEF-83 Verify the specific year and source for all numerical claims.

BRIEF-84 Interpretation.

BRIEF-85 Do not mix deaths, thoughts, attempts or age groups.

BRIEF-86 Survey methods, reporting completeness and data years differ.

BRIEF-87 Population associations do not predict an individual person's behavior.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

BRIEF-88 **8. Technology, Social Media and Website Design**

BRIEF-89 Digital opportunities.

BRIEF-90 A website can provide clear education, resource directories, accessible support navigation and links to professional help.

BRIEF-91 Digital risks.

BRIEF-92 Misinformation, privacy problems, inaccessible interfaces, harmful content and overconfident automated advice can undermine trust and safety.

BRIEF-93 Design principles.

BRIEF-94 Use calm, readable typography, plain language, mobile accessibility, strong contrast, visible human support options and minimal collection of sensitive data.

BRIEF-95 AI boundaries.

BRIEF-96 AI can help organize educational content but should never claim to diagnose, replace a therapist or serve as the only crisis response.

BRIEF-97 Have clinicians review crisis-related flows.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

BRIEF-98 **Website Copy and Agent Implementation Brief**

BRIEF-99 These are three distinct visitor pathways.

BRIEF-100 Keep language gentle, nonjudgmental, and brief.

BRIEF-101 The first two are supportive reflection tools, not clinical assessments.

BRIEF–102 **Pathway A — Navigate Your Thoughts**

BRIEF–103 Heading: Not sure what you're feeling?

BRIEF–104 Start here.

BRIEF–105 Website-ready copy: You don't need to have the right words.

BRIEF–106 *same as SURE-4:* Take a moment to notice what is on your mind.

BRIEF–107 *same as SURE-5:* You can explore your feelings at your own pace, without pressure or judgment.

BRIEF–108 *same as SURE-6:* If you need support, there are people ready to listen.

BRIEF–109 • Optional prompt: "What feeling is strongest right now?" with options such as overwhelmed, sad, anxious, numb, unsure.

BRIEF–110 • Offer a gentle check-in and the choice to pause, skip or speak with someone.

BRIEF–111 • Provide a visible "Get support now" link throughout.

BRIEF–112 **Pathway B — Work Through Your Thoughts**

BRIEF–113 Heading: A little space to reflect, one step at a time.

BRIEF–114 Website-ready copy: When thoughts feel overwhelming, it may help to slow down and focus on what you need in this moment.

BRIEF–115 Try naming what is difficult, what might help you feel supported, and one small, safe next step.

BRIEF–116 You do not have to handle everything alone.

BRIEF–117 • Invite optional journaling: "What has been weighing on you?"

BRIEF–118 • Offer grounding choices such as noticing surroundings, taking a break, or reaching out to a trusted person.

BRIEF–119 • End with a choice to save privately, clear the entry, or view support resources.

BRIEF–120 **Pathway C — Learn About Suicide**

BRIEF–121 Heading: Understand the facts.

BRIEF–122 Recognize the signs.

BRIEF–123 Find ways to help.

BRIEF–124 Website-ready copy: Suicide is a complex public health issue influenced by mental health, life experiences, relationships, and access to support.

BRIEF–125 Learning about warning signs, risk factors, prevention, and treatment can help reduce stigma and encourage compassionate conversations.

BRIEF–126 Support and recovery are possible.

BRIEF–127 • Create short, clearly titled educational modules: Understanding Suicide; Risk and Protective Factors; Warning Signs; Prevention; Treatment; Helping Someone Else.

BRIEF–128 • Cite WHO, CDC, NIMH and SAMHSA; show last-reviewed dates.

BRIEF–129 • Keep crisis support visible and avoid graphic or sensationalized content.

BRIEF–130 **Universal safety and product requirements**

BRIEF–131 • Show 988 (U.S. call/text) and an emergency-services pathway in a consistently visible location; adapt resources by visitor region.

BRIEF–132 • Do not require registration, payment or completion of a questionnaire to access support.

BRIEF–133 • Make all reflective questions optional.

BRIEF-134 • Provide clear Skip, Back, Exit and Clear options.

BRIEF-135 • Do not present automated risk scores as diagnosis, or imply the website is monitored by live clinicians unless it truly is.

BRIEF-136 • Avoid collecting sensitive journal text by default; explain retention, deletion and confidentiality limitations plainly.

BRIEF-137 • Have qualified mental health professionals review wording, crisis escalation pathways and updates before launch.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

BRIEF-138 **Primary Sources and Verification**

BRIEF-139 • World Health Organization (WHO), Suicide fact sheet: https://www.who.int/news-room/fact-sheets/detail/suicide

BRIEF-140 • Centers for Disease Control and Prevention (CDC), Suicide Prevention: https://www.cdc.gov/suicide/

BRIEF-141 • National Institute of Mental Health (NIMH), Suicide Prevention: https://www.nimh.nih.gov/health/topics/suicide-prevention

BRIEF-142 • Substance Abuse and Mental Health Services Administration (SAMHSA), Warning Signs: https://www.samhsa.gov/mental-health/suicidal-behavior/warning-signs

BRIEF-143 • 988 Suicide & Crisis Lifeline: https://988lifeline.org/

BRIEF-144 Editorial note: This document consolidates the earlier eight-topic research into an agent-friendly reference rather than reproducing every sentence verbatim.

BRIEF-145 It omits unverified 2026 figures and should undergo clinical and factual review before publication.

Decision: ☐ OK as written ☐ OK with changes ☐ Not for the website

Notes:

Tracker for the brief

Fill this in during or after the therapist's review. "Website use" says where the section would appear; nothing is published from this note yet.

Section	Website use	Reviewer status	Reviewer notes
1. Understanding suicide	Research	Not reviewed	
2. Causes and risk factors	Research	Not reviewed	
3. Warning signs	Help Someone	Not reviewed	
4. Effects on families, schools, communities	Research, Help Someone	Not reviewed	
5. Evidence-based prevention	Research	Not reviewed	

Section	Website use	Reviewer status	Reviewer notes
6. Treatment and recovery	Get Help	Not reviewed	
7. Statistics	Research	Not reviewed	Recheck every number before publishing
8. Technology and website design	Project guidance, not page copy	Not reviewed	
Pathway A: Navigate your thoughts	New interactive page, needs a product decision	Not reviewed	
Pathway B: Work through your thoughts	New interactive page, needs a product decision	Not reviewed	
Pathway C: Learn about suicide	Research	Not reviewed	
Universal safety requirements	Whole site; compare with SAFETY	Not reviewed	

Statuses: Not reviewed, Approved, Approved with changes, Not for the website. Also record the reviewer's role and the review date, never their personal contact details.

Part 4: Crisis resources and what each button does

An AI assistant read these on each provider's page. No person has confirmed them. No call, text, or chat was placed.

Crisis resources

"Read on" means Claude read the provider's own page in a browser on that date, without calling, texting, chatting, or submitting anything. It does not mean a person has confirmed it. A founder should read each page and fill in the last column. Recheck close to publication and at least monthly (proposed).

Resource	How to reach it	Official page	Read on	Person confirmed
988 Suicide & Crisis Lifeline	Call or text 988; chat at chat.988lifeline.org. 24/7	https://988lifeline.org/get-help/	Nov 6, 2026	
988 in Spanish	Call 988, then press 2; text AYUDA to 988; chat at chat.988lifeline.org/?lang=es	https://988lifeline.org/get-help/	Nov 6, 2026	
988 in other languages	Call 988 and ask for an interpreter or say the language. More than 240 languages, voice only	https://988lifeline.org/faq/calling-the-988-lifeline/faq-is-the-988-lifeline-available-in-other-languages-for-non-english-speakers/	Nov 6, 2026	
988 for Deaf and hard of hearing people	Dial 988 from a videophone; TTY users use a relay service or dial 711, then 988; text; chat	https://988lifeline.org/deaf-hard-of-hearing-hearing-loss/	Nov 6, 2026	
988 for LGBTQ+ youth (see the note below)	Call 988, then press 3; text PRIDE to 988	https://988lifeline.org/get-help/what-to-expect/	Nov 6, 2026	
Veterans Crisis Line	Dial 988, then press 1; text 838255; chat	https://www.veteranscrisisline.net	Nov 6, 2026	
Crisis Text Line	Text HOME to 741741. Free on AT&T, T-Mobile, Sprint, and Verizon; standard text rates on other carriers	https://www.crisistextline.org/about-us/faq/	Nov 6, 2026	
The Trevor Project (LGBTQ+ young people)	Call 1-866-488-7386; text START to 678-678; chat; TTY 1-866-803-3699	https://www.thetrevorproject.org/get-help/	Nov 6, 2026	
SAMHSA National Helpline (referrals, not a crisis line)	1-800-662-4357; TTY 1-800-487-4889. English and Spanish, 24/7	https://www.samhsa.gov/find-help/helplines/national-helpline	Nov 6, 2026	
FindTreatment.gov (treatment finder)	Website	https://findtreatment.gov	Nov 6, 2026	
Emergency services	Call 911. Text-to-911 works only in some places; call if you can	https://www.fcc.gov/consumers/guides/what-you-need-know-about-text-911	Nov 6, 2026	

On the website as a draft, a person must confirm

- **988 LGBTQ+ youth option.** 988's Get Help page says the service has been restored, and its What to Expect page lists press 3 on a call and PRIDE to 988 by text. The Trevor Project and a U.S. House statement date its return to September 30, 2026. SAMHSA's FAQ (December 2025) and performance page (October 2026) do not mention it, and no official page gives its age range or chat route. The Trevor Project is not part of it, so the Get Help page lists the two apart. CONTENT-REVIEW asks the reviewer about this.

Left off the website until a person confirms

- **Crisis Text Line in Spanish.** The provider's pages disagree on the keyword (HOLA or AYUDA).
- **Veterans Crisis Line TTY number.** Older VA pages list a 10-digit TTY number. The current call page says to use a relay service or dial 711, then 988.

Only a person can check

- That the Call and Text links open the dial and compose screens on a real iPhone and Android phone, with the right number filled in. Stop there and cancel.
- That the text links behave as expected with no message filled in. They open a blank message, so the person has to type the keyword (AYUDA, PRIDE, START, or HOME).
- The recorded menu on a 988 call. Nobody here has listened to it live, and it must not be tested by calling. Ask 988 or listen to the audio on its website.

What each control does

"Link format only" means nobody has tapped it on a phone. The phone test for all of these: tap, stop at the dial or compose screen, and cancel.

Control	Where	Goes to	Provider	Checked	Needs a person
Call 988	Crisis strip, buttons, closing banner, Help Someone urgent note	`tel:988`	988 Lifeline	Link format only	Phone test
Text 988	Crisis strip, buttons, closing banner, Help Someone urgent note	`sms:988`	988 Lifeline	Link format only	Phone test
Call 911	Crisis strip, Get Help, Help Someone urgent note	`tel:911`	Local 911	Link format only	Phone test. Decide whether it should be a link at all
Chat Online	Buttons, closing banner	chat.988lifeline.org	988 Lifeline	Page opens; form not submitted	Open on a phone
Ayuda en español	Every page	Spanish block on Get Help	Stillwater	Tested in browser	Spanish reviewer
Llamar al 988	Get Help, Spanish block	`tel:988`	988 Lifeline	Link format only	Phone test, Spanish reviewer

Control	Where	Goes to	Provider	Checked	Needs a person
Enviar AYUDA al 988	Get Help, Spanish block	`sms:988`, message blank	988 Lifeline	Link format only	Phone test, Spanish reviewer
Chatear en español	Get Help, Spanish block	chat.988lifeline.org/?lang=es	988 Lifeline	Page opens in Spanish; form not submitted	Spanish reviewer
988lifeline.org	Fallback line	988lifeline.org	988 Lifeline	Page opens	None
Confidentiality statement	Get Help	988's page	988 Lifeline	Page opens	None
988 options for Deaf and hard of hearing people	Get Help	988's page	988 Lifeline	Page opens	Deaf reviewer or Deaf-led organization
Call 988 (other languages)	Get Help	`tel:988`	988 Lifeline	Link format only	Phone test
Call 988, then press 1	Get Help	`tel:988`	Veterans Crisis Line	Link format only	Phone test
Text 838255	Get Help	`sms:838255`	Veterans Crisis Line	Link format only	Phone test
Veterans Crisis Line chat	Get Help	VCL chat page	Veterans Crisis Line	Page opens	None
Call 988, then press 3	Get Help	`tel:988`	988 LGBTQ+ youth service	Link format only	Phone test. Confirm the option with 988
Text PRIDE to 988	Get Help	`sms:988`, message blank	988 LGBTQ+ youth service	Link format only	Phone test. Confirm the option with 988
Call 1-866-488-7386	Get Help	`tel:+18664887386`	The Trevor Project	Link format only	Phone test
Text START to 678-678	Get Help	`sms:678678`, message blank	The Trevor Project	Link format only	Phone test
Trevor Project chat and TTY	Get Help	Trevor's help page	The Trevor Project	Page opens	None
Text HOME to 741741	Get Help	`sms:741741`, message blank	Crisis Text Line	Link format only	Phone test
Crisis Text Line website	Get Help	crisistextline.org	Crisis Text Line	Page opens	None
1-800-662-4357	Get Help	`tel:+18006624357`	SAMHSA	Link format only	Phone test
1-800-487-4889 (TTY)	Get Help	`tel:+18004874889`	SAMHSA	Link format only	Phone test
FindTreatment.gov	Get Help	findtreatment.gov	SAMHSA	Page opens	None

Control	Where	Goes to	Provider	Checked	Needs a person
A form (PDF)	Get Help	Stanley-Brown safety plan PDF	Its developers	File loads	See the question on permission
How this works on different platforms	Help Someone	988's social media page	988 Lifeline	Page opens	None
Our web host logs visitors' IP addresses	Home	Privacy line in the footer	Stillwater	Tested in browser	Lawyer question
GitHub says it logs and stores visitors' IP addresses	Every footer	GitHub's documentation	GitHub	Page opens	Lawyer question
Get Support Now	Header	Crisis options on the same page	Stillwater	Tested in browser	Founders: is the label right?
Quick exit / Salida rápida	Every page, bottom right	https://www.weather.gov/	Stillwater	Tested in browser: Back skips the page that was left, but can still reach earlier Stillwater pages	Reviewer: label, destination, and what it can't hide
Save 988 to your contacts / Guarda el 988 en tus contactos	Get Help, For Teens, Wallet card, About, and their Spanish twins	`assets/988-lifeline.vcf`, `assets/988-linea-es.vcf`	Stillwater	File checked. See `notes/worklogs/2026-11-06-claude-first-version.md` for phone tests	Save on a real iPhone and Android phone
Llamar al 988 (marca 2) / Enviar AYUDA al 988 / Chatear en español	Spanish pages	`tel:988`, `sms:988`, chat.988lifeline.org/?lang=es	988 Lifeline	Link format only	Phone test; fluent reader
Menu	Header, phones and tablets	Opens the page list	Stillwater	Tested in browser	Real phone with a screen reader
Three homepage cards	Home	Get Help, Help Someone, Research	Stillwater	Tested in browser	None
Show sources	Get Help, Help Someone	Opens the source list	Stillwater	Tested in browser	None

Text and chat with a crisis service start with automated steps. Texting asks you to accept terms and answer a few questions. Chat asks for a zip code, whether you are having thoughts of suicide, and how upset you feel. The Get Help page says so.

Part 5: Questions written for you

These are suggestions from an AI assistant, not decisions. Use the blank lines, or write on the notes pages at the end.

Wording is proposed. Nothing here is clinical advice.

Crisis wording on every page

- "Need help now?" replaced "In crisis?", and "Not sure what you need?" replaced "Not in crisis?", so a distressed visitor is not asked to judge their own risk. Are these better?
- Should 911 be a tap-to-call link in the pinned strip? Phones ask before calling, but an accidental tap is possible, and some people fear a police response.
- The header's "Suicide prevention & support" tagline and "Get Support Now" button both come from the supplied design. Both read as if Stillwater provides support, and the button doesn't say who does. Claude's suggestion: "Find suicide prevention support" and "Call or text 988".
- The text links open a blank message (`sms:741741`, `sms:678678`, and `sms:988` for AYUDA and PRIDE), so a person in distress has to remember the keyword. The label on the button is all that reminds them. Is that acceptable?
- Is the privacy wording right? The homepage says "Stillwater has no sign-ups, forms, or tracking. Our web host logs visitors' IP addresses." Each footer says "no sign-ups, forms, analytics, or cookies" and links GitHub's statement. A lawyer should read this too.
- Footers and draft notes say an AI assistant drafted the content and compared numbers and links with each provider's website. Is that the right way to say it, and should it stay after review?

Get Help

- Is it right to tell people that 988 counselors may contact emergency services, with 988's own figure of fewer than 2 in 100 calls? The page also says, from 988's terms, that you may not get a say, and that the terms ask you to call 911 first for an immediate threat. It is honest, but could it put someone off calling?
- Should the page list the 988 LGBTQ+ youth option (press 3, PRIDE to 988), and how? 988's own pages list it, none states an age range or chat route, and SAMHSA's older FAQ doesn't mention it. The Trevor Project is not part of it, so the page lists them apart. See SAFETY.
- Is the description of a safety plan suitable for someone making one alone? The developers present it as made with a clinician, and evidence for teens using one alone is thin. See the Research page.
- The Stanley-Brown developers require written permission to reword their form or put it into a website. The page describes safety plans in general and links to their PDF. Is that enough? Should the project ask them?
- Should the page add more about shared devices and phone bills, or a quick-exit button?

Under 18

- What should minors be told about confidentiality? The page says conversations are confidential with limits. It tells them to expect a school counselor to tell a parent or guardian, because the American School Counselor Association calls that "non-negotiable", even at 18. The exception is when a parent's abuse or neglect is the reason, when the counselor contacts child protective services. Could this wording put a teen off telling a counselor?
- 988's terms mention insurance statements that could reach a parent when a center bills insurance. The page says so. Is that the right level of detail?
- What should a teen do if no trusted adult is available, or home is not safe?

- A lawyer should look at minors' privacy and the under-13 question.

Help Someone

- The urgent note doesn't follow SAMHSA's order. SAMHSA's Suicide Prevention page, read November 6, lists calling 911 if danger seems imminent, then calling or texting 988, and further down staying with the person or making sure another caring person is there. The note puts staying second and 988 last. "If it is safe" is Claude's addition, not SAMHSA's. Is the order right, and should that phrase stay? No general official page describes calling 988 together, so the page no longer tells people to.
- The warning signs follow NIMH's 2025 list with no urgent tier, because official sources sort urgent signs differently. Is that right?
- Sources differ on when not to leave someone alone. The general rule on the page follows NIMH: when they say they are going to end their life. The National Association of School Psychologists sets a much lower bar for young people showing signs, with constant supervision. The parent paragraph sits between the two: stay close when a child has a plan or can't stop thinking about ending their life. Are these the right thresholds?
- The page keeps safety advice general and names no items: "remove or safely store anything they could use to hurt themselves". NIMH, SAMHSA, 988, and the school psychologists all tell helpers to remove means. Is it specific enough? The page's advice is from those sources, and the Research page has no study of steps at home. One review turned up during checking, Spitzer 2024 (PMID 38324739, not read in full), reports most studies of counseling families on this at high risk of bias.
- Some sources listed on this page, such as the VA caregiver toolkit, name specific items. Nobody has checked every source link for this. Should they stay linked?
- For a young person whose friend told them by text or online, the page says to keep talking if you can while you get an adult, or call or text 988. That wording is Claude's, because the sources don't cover it. Is it right?

Research

- Are the findings fair? Each has a "What it can't show" line. Please check every number against its source.
- Is it right to include the study on news coverage of celebrity deaths? It is worded without figures and says it does not apply to talking with someone.
- Two papers are cited without links because they name methods. Should they be linked? Open these knowing that: Steeg 2025, https://pmc.ncbi.nlm.nih.gov/articles/PMC12699607/, and the BMJ 2020 review, https://pmc.ncbi.nlm.nih.gov/articles/PMC7190013/. The other research and source links were not all checked for this.
- The page dropped a figure: about 4 in 10 interviewed callers had suicidal thoughts again by the interview, about two weeks later on average (Gould 2025, 40.8%). A figure with no step can read as "calling doesn't last", so the page says "Some people had suicidal thoughts again later" and says they can reach out again. Was that right?
- Albaum 2025: the printed result for suicidal thoughts (Hedges g 0.11, 95% CI 0.01 to 0.21) excludes zero and sits on the side favoring the comparison group, while the authors call it not significant. Figure 2A shows the same. The page says a benefit has not been shown and that the studies can't settle small effects in either direction. Please read the figure, and consider asking the authors.
- Steeg 2025: evidence is weak for one policy area studied mainly in the U.S. The page only says strength varies by approach. Should it say more, for a U.S. site?
- Studies of messages alone are mixed, including Witt 2021, Skopp 2023, and Reger 2024 and 2026. The page gives one such trial (Comtois). Should it say more?

- Evidence moved recently: the caring messages trial was published November 2, 2026, and the 988 mortality paper has published letters and a reply. Recheck before release.

Spanish

- A fluent Spanish speaker should check the Spanish block on Get Help. Claude wrote it from 988's English and Spanish pages.

Built at one founder's request on November 6, 2026. Wording is proposed.

- **Right now box (B1).** Get Help now opens with four short steps, and four long sections fold behind "Show more". Every line repeats text already on the page. Are the steps right, and is anything folded that someone in crisis needs to see at once? "Other ways to reach someone" stays open on purpose.
- **Quick exit (B2).** It opens weather.gov and replaces only the current page in the tab's history. Back can still reach Stillwater pages opened earlier in that tab. It can't clear earlier history, other tabs, or saved offline copies. The About pages say so; should the button itself say more? Is the label right, and is weather.gov a good neutral destination?
- **Offline copies (A6).** After one visit, the site's pages are saved on the device so the numbers load without a signal. On a shared device that is a trace of the visit. Is the benefit worth it, and should the privacy note mention it?
- **Spanish (B6).** An AI translated Home, Get Help and Help Someone. A fluent reader should check every line, and the reviewer should decide whether the wording should avoid masculine-only forms such as "No estás solo" and "estés listo". The pages use "tú", like 988's own Spanish pages. Texting steps in Spanish are described as possibly different from English, because no one has seen them.
- **Myths and facts (D1),** at the top of Research. Each heading states the fact, and the myth comes second, because repeating a myth as a headline can make it stick. Is that the right format? Is "Most 988 calls don't involve emergency services" helpful, or could it read as a promise?
- **Wallet card and poster (D2).** The printed text comes only from the SAFETY table. Is the selection right (988, Spanish, veterans, The Trevor Project, Deaf and hard of hearing, 911)? Should the card name Stillwater at all?
- **For Teens (D3).** Built before ages are decided. Every paragraph repeats text from Get Help or Help Someone. What should a teen read first? What if no adult feels safe? A lawyer should look at the privacy wording for minors.
- **Long sentences (C4).** `python3 tools/check_readability.py` lists sentences over 25 words. Most explain when 988 may involve emergency services. SHORTER-SENTENCES has proposed rewrites. The pages haven't changed. Can they be shortened without losing accuracy?

Built on November 6, 2026, at one founder's request, on branch `site/first-version-build`. The other founder hasn't agreed yet. Wording is proposed.

- **Not sure what you're feeling? (B3).** The page has no typing, saving, or scores: a person taps to open short notes on five feelings. The heading and opening come from Pathway A in the brief. Pathway B's journal was not built. 988 and 911 come first on the page. Is a reflection page right for someone in crisis at all? Should "Numb or empty" send people to 988 this directly? The page avoids NIMH's diagnostic definitions, and keeps only its advice to talk to a provider if a low mood doesn't go away. Is that right?
- **After a loss (B4).** "It's not your fault" is a heading. TAPS says it, and the CDC says suicide has no single cause, but someone may feel the site can't know their situation. Is the wording right? The list of feelings is AFSP's, including relief and thoughts of suicide. Should it say more about thoughts of suicide after a loss? The trauma paragraph sits inside a fold on purpose, with no details of any death. Is that right? The page lists

outside groups that Stillwater hasn't vetted, and AFSP and SAVE say they don't vet groups either. Someone with lived experience of loss should read this page.

- **Save 988 to contacts (A5).** A contact on a shared phone can be seen by others, and the page says so. Should the button appear on the teen page?
- **About and privacy.** It says plainly that nobody at Stillwater watches the site and there is no way to send a message. A lawyer should read this page too.
- **Quick exit limits.** Testing showed that Back can still reach earlier Stillwater pages in the same tab, because only the current page is replaced. The About pages now say so. An earlier note said Back doesn't return to Stillwater, which was only true when one page had been opened.
- **For Teens, from an independent AI check.** "Its rules don't set a minimum age" rests on finding no age limit in 988's terms and privacy policy, not on 988 saying so. "It is separate from The Trevor Project" has no source on the page; SAFETY explains where it comes from. Telling a teen to expect a school counselor to tell a parent is accurate, but could deter them. Should it say why, and that 988 stays available?
- **Spanish Research page.** It was translated before the English version was reviewed, at a founder's request, so that the Spanish pages are complete for review. If the English changes, the Spanish must change too.

Questions about the research brief

These come from comparing the brief with the current site and SAFETY. They are suggestions, not decisions.

- **Pathways A and B are interactive tools, not just text.** Pathway B offers to "save privately" a journal entry. The site currently collects nothing (SAFETY rule 5), and the brief itself says to avoid collecting journal text by default. Should Pathway B work only on the visitor's own device with nothing sent anywhere, or wait until the founders decide about saved data?
- **Is a reflection tool appropriate for someone in crisis?** Should Pathways A and B show 988 and 911 before the first question, not only alongside it?
- **Minors.** Section 3 says to involve "a trusted adult for young people." What should a young person do if no adult feels safe? Supported ages are undecided.
- **Statistics (section 7).** The brief gives WHO's figure of more than 720,000 deaths a year (2021 estimate) and asks for it to be rechecked. Who rechecks the numbers, and how often?
- **Region.** The brief says to "adapt resources by visitor region." The site is U.S.-only. Should international visitors be pointed elsewhere?
- **Overlap with other drafts.** Branch `site/phone-accessibility-content` has separate draft text for all three pages with its own reviewer questions in `notes/CONTENT-REVIEW.md` on that branch. Review both together and pick one wording for each page.

Your answers

Part 6: How the research was read

Claude read these, not a person. "Summary tool" means a fetch tool that returns a summary instead of the raw page, so treat fine detail as second-hand. PubMed records came through NCBI's official service, because PubMed's web pages show a browser check. No block or browser check was bypassed.

Finding	Source	How it was read	Still unconfirmed
Many callers say the call helped	Gould 2025	Full text	
Studies of crisis lines are limited	Hoffberg 2020	Full text	
Early research since 988	Patel 2026	Summary tool, PubMed record	Fine detail
Early research since 988	Letters from Herring and Purtle, authors' reply	PubMed records, journal previews	Purtle and the reply showed only a preview
Asking does not raise thoughts	Polihronis 2022	Abstract; full text blocked	"Pooled 8 randomized trials" is likely right but unconfirmed. The abstract says 8 studies were pooled and, separately, that 8 RCTs gave the strongest evidence
Asking does not raise thoughts	McGuire 2026	Abstract	
Safety planning in adults	Nuij 2021	Full text, University of Glasgow copy; journal page blocked	
Safety planning in children and teens	Albaum 2025, corrected 2026	Full text and Figure 2; correction through summary tool	The question above
Follow-up after an attempt	Homan 2026	Full text	
Follow-up after an attempt	Comtois 2026	Summary tool, PubMed abstract for the main numbers. The journal page showed a browser check on November 6, not bypassed	Recruitment and control-care wording. A person should open it
Safer surroundings	Steeg 2025	Full text, method details not recorded	
Treatment	Witt 2021	Abstract and plain-language summary	
Treatment	Sim 2025	Abstract	
Treatment	Cottrell 2026	Abstract; full report blocked	Comparison groups, and the direction of its suicidal thoughts exception at 6 months
News coverage	BMJ 2020 review	Full text, method details not recorded	

Finding	Source	How it was read	Still unconfirmed
Stories of coping	Niederkrotenthaler 2022	Abstract; full text blocked	How soon after the stories outcomes were measured
What this page can't tell you	van der Burgt 2024	Abstract, full text searched for specific points	

How the first-version pages' sources were read

Claude read these on November 6, 2026, in a real browser where possible, and one separate AI session checked about 112 statements against them. Neither is human review. Not read in full: the AFSP booklets and toolkits, which are image-only flipbooks, so the pages only link to them. 988's old loss survivors page now answers "not found", so it isn't cited.

How the pages compare with the research brief

RESEARCH-BRIEF and the draft pages were written separately. **The page text did not come from the brief,** and nothing from the brief is on the website. Review them together and pick one wording for each page. This is Claude's comparison, for the reviewer.

Brief section	Closest draft on the pages	To settle
1. Understanding suicide	None. Help Someone and Research cover asking about suicide	Add definitions and myths?
2. Causes and risk factors	None	Add this? SAFETY says not to name a single cause
3. Warning signs	Help Someone, signs and five steps	The drafts follow NIMH's list, and the brief's list differs. Choose one
3. Urgent support	Crisis strip and buttons	Same direction
4. Families, schools, communities	Help Someone: child or teen, look after yourself	The drafts say nothing about grief after a suicide or support for schools afterward. Add this?
5. Evidence-based prevention	Research page	The brief covers levels and WHO's LIVE LIFE. The drafts cover single findings with limits
6. Treatment and recovery	Get Help: support over time, safety plans. Research: treatment	Same direction. The brief calls a safety plan collaborative
7. Statistics	None. The drafts use no population statistics	The brief's numbers need rechecking, and SAFETY asks for few
8. Technology and website design	Project guidance	Matches SAFETY rules
Pathways A and B	None. These are interactive tools and were not built	Product decision. Pathway B's saved journal conflicts with SAFETY rule 5
Pathway C	Research and Help Someone	The brief wants short modules and last-reviewed dates
Universal requirements	Crisis strip, SAFETY rules	The brief says to adapt by region, and the site is U.S.-only

Part 7: Proposed shorter sentences

Optional. These are AI-drafted rewrites of the longest sentences. The pages have not changed. Mark any you accept.

Get Help

- **Now (28 words):** The Lifeline's confidentiality statement lists when information can be shared, including when someone is at imminent risk of harm, when the law requires it, or when you agree. **Proposed:** The Lifeline's confidentiality statement lists the times when information can be shared. These include when someone is at imminent risk of harm, when the law requires it, or when you agree. **For the reviewer:** Keep the link on "confidentiality statement". "These include" keeps the list open, as "including" does: the statement also names other cases, such as court orders. Reviewer's choice: "imminent risk of harm" is the statement's own term. The proposal uses "right now or very soon", which is how this page already words the same idea under emergency services, but a reader could take "very soon" more broadly. To keep the exact term, write "at imminent risk of harm", which also stays under 25 words. The tool's rough estimate is still about grade 9, because long words like "confidentiality" stay. The Spanish page has a matching 37-word sentence.
- **Now (26 words):** On a shared phone or computer, visits to this site can show up in browser history, and calls or texts can show up in phone logs. **Proposed:** On a shared phone or computer, visits to this site can show up in browser history. Calls or texts can also show up in phone logs. **For the reviewer:** the same sentence is on teens/index.html, so change both or neither. "Also" keeps the second sentence under "On a shared phone or computer". about/index.html has a similar 20-word sentence that the tool doesn't flag. The Spanish page has a matching 32-word sentence.
- **Now (39 words):** The Lifeline recommends that counselors contact emergency services, which can mean 911, police, or a sheriff, only if someone is in danger of being hurt right now or very soon and they can't work out a safer plan together. **Proposed:** Emergency services can mean 911, police, or a sheriff. The Lifeline recommends that counselors contact them only if two things are true. First, someone is in danger of being hurt right now or very soon. Second, the counselor and the person who reached out can't work out a safer plan together. **For the reviewer:** Keeps "recommends", "can mean", "only if", and both conditions. "Two things are true" matches the Lifeline's FAQ, which joins the conditions with "and when", so both are needed. If accepted, the next sentence (HELP-67) must change from "It reports" to "The Lifeline reports", or its source is no longer clear. "They" is kept from the original and means the counselor and the person. For the reviewer, not changed here: "safer plan" is the page's existing wording for the FAQ's "a less invasive plan for the caller/texter's safety". The Spanish page has a matching 46-word sentence.
- **Now (47 words):** The Lifeline's terms say that if a counselor believes there is an immediate threat to someone's physical safety, you may not get a say: they may ask for 911 or a mobile crisis team to come to your location, and can share your information without your permission. **Proposed:** The Lifeline's terms say that if a counselor believes there is an immediate threat to someone's physical safety, you may not get a say. In that case, the counselor may ask for 911 or a mobile crisis team to come to your location. In that case, your information can also be shared with them without your permission. **For the reviewer:** "You may not get a say" is word for word and stays in one sentence with its condition. On its own it would carry more weight, and notes/CONTENT-REVIEW.md already asks whether this wording could put someone off calling. "In that case" repeats so neither later sentence reads as a general rule. "Under the Lifeline's terms" replaces "The Lifeline's terms say" to avoid "say ... a say". One wording change for accuracy: "they ... can share" became "can also be shared". The terms give the 911 or mobile crisis request to the counselor, but the sharing clause says "We reserve the right to disclose". For the reviewer, not added because they would be new to this sentence: the

terms describe a threat "requiring a rapid in-person response", and describe sharing with "authorities" such as 911, law enforcement, or mobile crisis teams, "in certain circumstances". The tool's rough estimate is still about grade 10, because words like "immediate" and "permission" stay. The Spanish page has a matching 52-word sentence.

- **Now (31 words):** A safety plan is a short written list of what to do when things get hard, made ahead of time so you don't have to work it out during a crisis. **Proposed:** A safety plan is a short written list of what to do when things get hard. It is made ahead of time, so you don't have to work out what to do during a crisis. **For the reviewer:** Uses "It is made" rather than "You make it": notes/CONTENT-REVIEW.md asks whether this description suits someone making a plan alone, and the next sentence says a professional can help. "Work it out" became "work out what to do" so "it" doesn't mean two things in one sentence. The Spanish page already splits this sentence the same way.
- **Now (28 words):** It can also help to tell an adult you trust, such as a parent or guardian, a relative, a teacher, a coach, a spiritual leader, or a doctor. **Proposed:** It can also help to tell an adult you trust. This could be someone like a parent or guardian, a relative, a teacher, a coach, a spiritual leader, or a doctor. **For the reviewer:** the same sentence is on teens/index.html, so change both or neither. All six examples are kept. "Someone like" keeps the list open, as "such as" does. The Spanish page has a matching 29-word sentence.

Help Someone

- **Now (28 words):** If they have a suicide plan or can't stop thinking about ending their life, call or text 988 right away, and call 911 if they need immediate help. **Proposed:** If they have a suicide plan or can't stop thinking about ending their life, call or text 988 right away. Also call 911 if they need immediate help. **For the reviewer:** Splits at 'and'. Both conditions, 'right away', and both numbers stay, and 'Also' keeps 911 as an added step, not a replacement for 988. For the reviewer to check: as its own sentence, the 911 line could be read as applying beyond the opening 'If...'. It also now sits between that 'If...' and the next sentence, 'Stay close and don't leave them alone while you get help', so a reader could tie 'Stay close' to the 911 case only. notes/CONTENT-REVIEW.md already asks about this paragraph's threshold for staying close. Review packet row SOME-81. If accepted, the parallel Spanish sentence needs the same split (es/ayudar-a-alguien/index.html, line 205). Rough reading grade: 10.1 before, 5.0 after. Whole page: this row and the next are the only flags, and a second pass over all visible text, footer included, found no others.
- **Now (38 words):** If you're worried about suicide, it's okay to ask directly: "Are you thinking about suicide?" Encourage your friend to talk to a trusted adult, such as a parent, coach, teacher, or school counselor, and talk to one yourself. **Proposed:** If you're worried about suicide, it's okay to ask directly: "Are you thinking about suicide?" Encourage your friend to talk to a trusted adult. This could be a parent, coach, teacher, or school counselor. Talk to a trusted adult yourself, too. **For the reviewer:** Optional for length. This is really two sentences, 15 and 23 words, so neither is over 25. The readability checker counts them as one because it doesn't end a sentence at a closing quote mark; the review packet already lists the second as its own row (SOME-96). The proposal makes the second one plainer and clears the flag: the checker counts 24, 10, and 7 words, where the 24 is the question plus 'Encourage your friend...' (really 15 and 9). The question itself is unchanged. 'This could be' keeps the list as examples, as 'such as' did, and 'a trusted adult' replaces 'one' so the last sentence is clear on its own. Total words go up from 38 to 41. If accepted, the same words on teens/index.html (line 184, review packet row TEEN-61) and the parallel Spanish sentence (es/ayudar-a-alguien/index.html, line 210) need matching changes. Rough reading grade after: 6.7.

For Teens

- **Now (28 words):** It can also help to tell an adult you trust, such as a parent or guardian, a relative, a teacher, a coach, a spiritual leader, or a doctor. **Proposed:** It can also help to tell an adult you trust. That could be a parent or guardian, a relative, a teacher, a coach, a spiritual leader, or a doctor. **For the reviewer:** "That could be" keeps the list as examples, as "such as" did. The same sentence is on get-help/index.html, so change both together. The Spanish version is on es/adolescentes/.
- **Now (26 words):** On a shared phone or computer, visits to this site can show up in browser history, and calls or texts can show up in phone logs. **Proposed:** On a shared phone or computer, visits to this site can show up in browser history. Calls or texts can also show up in phone logs. **For the reviewer:** "also" keeps the second sentence tied to "On a shared phone or computer". The same sentence is on get-help/index.html, and about/index.html words it almost the same way. The Spanish version is on es/adolescentes/.
- **Now (38 words):** If you're worried about suicide, it's okay to ask directly: "Are you thinking about suicide?" Encourage your friend to talk to a trusted adult, such as a parent, coach, teacher, or school counselor, and talk to one yourself. **Proposed:** If you're worried about suicide, it's okay to ask directly: "Are you thinking about suicide?" Encourage your friend to talk to a trusted adult, such as a parent, coach, teacher, or school counselor. Talk to one yourself, too. **For the reviewer:** The 38 is a counting quirk. The tool doesn't split after a question mark followed by a closing quote, so it counts two sentences (15 and 23 words) as one. Neither is really over 25. The proposal still splits the second so "talk to one yourself" stands on its own, giving 15, 18, and 5 words. The tool will still flag this text, at 33 words, unless its splitter changes. The same sentence is on help-someone/index.html, so change both together. The Spanish version is on es/adolescentes/.

Not sure what you're feeling?

- **Now (26 words):** NIMH lists a lasting "empty" mood among the signs of depression, and says some older adults notice a lack of emotions instead of a low mood. **Proposed:** NIMH lists a lasting "empty" mood among the signs of depression. It also says some older adults notice a lack of emotions instead of a low mood. **For the reviewer:** "It" means NIMH, so both claims stay attributed. A separate point, not changed here: someone who opens only "Numb or empty" doesn't see NIMH spelled out. The full name is only in the "Overwhelmed" section and the sources list. The Spanish version is on es/que-siento/.

After a loss

- **Now (27 words):** TAPS says the intense pain will not always feel the way it does right now, and that it can ease and soften with support and with time. **Proposed:** TAPS says the intense pain will not always feel the way it does right now. TAPS also says the intense pain can ease and soften with support and with time. **For the reviewer:** Repeating "TAPS" keeps both claims attributed. "can" stays "can", not "will", and both "with support" and "with time" stay. The Spanish version is on es/despues-de-una-perdida/.

About and privacy

- **Now (33 words):** On a secure connection, your browser may save a copy of this site's pages and files after your first visit, so the crisis numbers can still show with a weak signal or none. **Proposed:** On a secure connection, your browser may save a copy of this site's pages and files after your first visit. That way, the crisis numbers can still show with a weak signal or none. **For the reviewer:** "That way" keeps the link between the saved copy and the numbers showing. "may" and "can" stay, so it doesn't read as a promise, and the limits are still in the list right after it. The Spanish version is on es/acerca-de/. Separately, index.html and card/index.html have no sentences over 25 words, so they have no rows.

Research

- **Now (26 words):** The 988 Lifeline reports that fewer than 2 in 100 calls involve emergency services, and that more than half of those happen with the caller's consent. **Proposed:** The 988 Lifeline reports that fewer than 2 in 100 calls involve emergency services. It also reports that more than half of those cases happen with the caller's consent. These figures are not a guarantee. **For the reviewer:** The next sentence on the page, 'This is not a guarantee.', must stay right after this one. Please check it still reads as covering both figures; 'These figures are not a guarantee.' would make that clear. A near-identical sentence is on get-help/index.html, so change both together. Kept 'consent' because a plainer word could change the meaning. For all rows: es/investigacion/index.html has matching Spanish sentences that would need the same change.
- **Now (27 words):** A review that pooled 8 randomized trials found no significant increase in suicidal thoughts or behavior, self-harm, or distress among people who were asked about these topics. **Proposed:** A review combined the results of 8 randomized trials. It found no significant increase in suicidal thoughts or behavior, self-harm, or distress among people who were asked about these topics. **For the reviewer:** 'Pooled' became 'combined the results of'; the page already says 'combined' for another review. Kept 'randomized' and 'significant' because they carry the study's meaning, so this still reads above 6th grade (rough estimate 9.5). notes/CONTENT-REVIEW.md lists 'pooled 8 randomized trials' as likely right but unconfirmed. This rewording makes the same claim, so it still needs checking.
- **Now (26 words):** A 2026 experiment with 700 U.S. teens also found that anonymous suicide questionnaires did not raise the desire to self-injure or to die right afterward. **Proposed:** A 2026 experiment also tested this with 700 U.S. teens. It found that anonymous suicide questionnaires did not raise the desire to self-injure or to die right afterward. **For the reviewer:** 'Right afterward' stays because it is a limit; the page also says this study measured only immediate effects. 'It also found' keeps the link back to the review in the sentence before.
- **Now (29 words):** A review of 6 studies in adults found that people who received a safety planning-type intervention had a lower risk of later suicidal behavior than people in comparison groups. **Proposed:** A review looked at 6 studies in adults. It found that people who received a safety planning-type intervention had a lower risk of later suicidal behavior than people in comparison groups. **For the reviewer:** Kept the technical term 'safety planning-type intervention', so this still reads above 6th grade (rough estimate 10.6). The next paragraph calls these 'programs', so 'program' is an option if the reviewer agrees it means the same.
- **Now (35 words):** What it can't show: Four of the 6 studies were rated at high risk of bias, the pattern of results suggests some less favorable studies may be missing, and no children or teens were included. **Proposed:** What it can't show: Four of the 6 studies were rated at high risk of bias. The pattern of results suggests some less favorable studies may be missing. No children or teens were included. **For the reviewer:** Split into three with no change in meaning. The bold label 'What it can't show:' stays and covers all three sentences.
- **Now (28 words):** A review of brief programs built around a safety plan for children and teens found no clear benefit, based on 10 studies, 5 of which could be pooled. **Proposed:** A review looked at brief programs built around a safety plan for children and teens. It found no clear benefit, based on 10 studies. The results of 5 of those studies could be combined. **For the reviewer:** 'Pooled' became 'combined'. 'No clear benefit' and the counts of 10 and 5 studies are unchanged. The open question in notes/CONTENT-REVIEW.md about this paper's printed result for suicidal thoughts is not affected.
- **Now (27 words):** A separate 2026 trial with 709 American Indian and Alaska Native adults at high suicide risk added culturally adapted caring messages to help connecting with local resources. **Proposed:** A separate 2026 trial included 709 American Indian and Alaska Native adults at high suicide risk. It added culturally adapted caring messages on top of help connecting with local resources. **For the reviewer:** 'To help

connecting' became 'on top of help connecting', so it can't be read as 'in order to help'. The Spanish page already reads this way. notes/CONTENT-REVIEW.md lists this paper's recruitment and control-care wording as unconfirmed, and this sentence holds both, so a person should check it against the paper. The group names and 'culturally adapted' keep this above 6th grade (rough estimate 10.7).

- **Now (28 words):** An overview of 20 research reviews found good support for making dangerous items and places harder to reach across whole populations, with several approaches that appear to work. **Proposed:** An overview looked at 20 research reviews. It found good support for making dangerous items and places harder to reach across whole populations, with several approaches that appear to work. **For the reviewer:** Keeps the limit 'across whole populations' and the hedge 'appear to work'. No method is named. Rough reading estimate is 7.9.
- **Now (27 words):** Another analysis of 26 trials in teens found that no one therapy worked better than the care the other group received, for repeat self-harm at 12 months. **Proposed:** Another analysis looked at 26 trials in teens. For repeat self-harm at 12 months, it found that no single therapy worked better than the care the other group received. **For the reviewer:** 'No one therapy' became 'no single therapy', so 'no one' can't be misread as 'nobody'. 'That doesn't mean care doesn't help.' must stay right after this. notes/CONTENT-REVIEW.md lists this paper's comparison groups as unconfirmed, and this rewording makes the same claim, so it still needs checking.
- **Now (33 words):** People who saw it did rate themselves as more likely to contact the helpline later, but the study could not check whether they did (van der Burgt and colleagues, JMIR Mental Health, 2024). **Proposed:** People who saw it did rate themselves as more likely to contact the helpline later. But the study could not check whether they did (van der Burgt and colleagues, JMIR Mental Health, 2024). **For the reviewer:** Split only. The citation keeps its link and stays at the end, because it also covers the sentence before this one.

Part 8: Decisions and sign-off

Please record your role and the date, such as "licensed clinical social worker, November 2026". Please don't write your name, license number, or contact details here. The founders will keep any such record outside the project.

The website

Section	Approved	Approved with changes	Not for the website	Reviewer's role and date
Home	☐	☐	☐	
Get Help	☐	☐	☐	
Help Someone	☐	☐	☐	
For Teens	☐	☐	☐	
Not sure what you're feeling?	☐	☐	☐	
After a loss	☐	☐	☐	
Research	☐	☐	☐	
Wallet card and poster	☐	☐	☐	
About and privacy	☐	☐	☐	
Page not found	☐	☐	☐	
Text on every page (Part 1)	☐	☐	☐	

The research brief

Section	Approved	Approved with changes	Not for the website	Reviewer's role and date
Brief: 1. Understanding suicide	☐	☐	☐	
Brief: 2. Causes and risk factors	☐	☐	☐	
Brief: 3. Warning signs	☐	☐	☐	
Brief: 4. Effects on families, schools, communities	☐	☐	☐	
Brief: 5. Evidence-based prevention	☐	☐	☐	
Brief: 6. Treatment and recovery	☐	☐	☐	
Brief: 7. Statistics	☐	☐	☐	
Brief: 8. Technology and website design	☐	☐	☐	
Brief: Pathway A: Navigate your thoughts	☐	☐	☐	

Section	Approved	Approved with changes	Not for the website	Reviewer's role and date
Brief: Pathway B: Work through your thoughts	☐	☐	☐	
Brief: Pathway C: Learn about suicide	☐	☐	☐	
Brief: Universal safety requirements	☐	☐	☐	

Overall

Is this site safe to show the public with the changes you asked for? ☐ Yes ☐ Yes, after the changes are made and I have seen them ☐ No ☐ Not my call

Anything else the founders should know, including who else should review this:

Reviewer's role:

Date:

For the founders, after the review

- Copy each result into "Review records" in `notes/CONTENT-REVIEW.md`, and the tracker in `notes/RESEARCH-BRIEF.md`. Use the role and date only.
- Make the changes on a task branch, and show the reviewer anything that changed a safety message.
- Update the review list on the About pages. Remove a page's draft note only after its review is recorded.
- Keep any signed copy with personal details outside this public project.

Notes